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DEPARTMENT OF THE AIR FORCE INSTRUCTION 44-172

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Medical

MENTAL HEALTH READINESS

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This Department of the Air Force Instruction (DAFI) implements Air Force Policy Directive (AFPD) 44-1, *Medical Operations*. It incorporates requirements outlined in DoDI 6490.08, *Command Notification Requirements to Dispel Stigma in Providing Mental Health Care to Service Members*; DoDI 6490.09, *DoD Directors of Psychological Health*; DoDI 6490.05, *Maintenance of Psychological Health in Military Operations;* DoDI 6200.03, *Public Health Emergency Management Within the Department of Defense;* and DoWI 6490.10, *Continuity of Mental Health Care for Transferring and Separating Service Members*. This instruction provides guidance for Mental Health (MH) personnel as it pertains to MH readiness of Department of Air Force (DAF) personnel. This instruction applies to all active-duty (AD) United States Space Force (USSF), Regular Air Force (RegAF), Air Force Reserve (AFR), and Air National Guard (ANG) personnel whenever eligible for DoW medical services. The Air Reserve Component (ARC) does not maintain separate systems to provide MH treatment, including substance use disorder treatment. Clarification about ARC-specific policies, processes, and/or procedures should be directed to Headquarters Air Force Reserve Command (AFRC)/SG MH Consultant at Robins AFB, GA or ANG/SG Medical Service Office at Joint Base Andrews, Maryland.

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SUMMARY OF CHANGES

This document has been substantially revised and must be completely reviewed. This publication merges AFI 44-172, *Mental Health* and AFI 44-153, *Disaster Mental Health,* which is now rescinded. While clinical standards of care and treatment protocols in the Medical Treatment Facilities (MTFs) are executed under Defense Health Agency (DHA) authority, determinations regarding a service member's (SM's) operational suitability, duty limitations, and deployability remain strictly DAF command decisions. MH providers will leverage Air Force Surgeon General (AF/SG) and Air Force Medical Command (AFMEDCOM) guidance to provide readiness focused medical recommendations; however, commanders retain the ultimate authority to assume operational risk and make final duty determinations.

Chapter 1—MENTAL HEALTH OVERVIEW **6**

1.1. Overview. 6

1.2. Mission. 6

1.3. Vision. 6

1.4. Scope. 6

Chapter 2—ROLES AND RESPONSIBILITIES **7**

2.1. Air Force Surgeon General (AF/SG). 7

2.2. Air Force Medical Command (AFMEDCOM) Commander. 7

2.3. USAF Judge Advocate General (AF/JA). 7

2.4. Air Force Director of Psychological Health (AF/DPH). 7

2.5. USSF/DPH. 8

2.6. AFMEDCOM MH Chief. 9

2.7. AF/SG MH CFMs/Consultants. 9

2.8. MRC DPH. 9

2.9. MAJCOM/SG. 10
2.10. MAJCOM DPH/Combatant Command DPH (CCMD/DPH). 10
2.11. 4C0X1 MAJCOM Functional Manager (MFM) or equivalent. 11
2.12. Mental Health Advisory Board (MHAB). 11
2.13. Installation Commander. 11
2.14. NGB DPH will refer to ANG policy and directives. 12
2.15. Air Force Reserve Installation DPH (AFR IDPH). 12
2.16. Installation Staff Judge Advocate (SJA). 12
2.17. MDG Commander and Reserve Medical Group Commander or equivalent. 12
2.18. Chief of Medical Staff (SGH). 13
2.19. Chief of Aerospace Medicine (SGP). 13
2.20. Squadron Commander Responsible for MH Flight. 13
2.21. MH Flight Commander (or equivalent). 14
2.22. MH Flight Chief. 15
2.23. Installation DPH (IDPH). 15
2.24. MH Flight Medical Director. 16
2.25. MH Element Chief. 16
2.26. Alcohol and Drug Abuse Prevention and Treatment (ADAPT). 17
2.27. FAP Manager Element Chief. 17
2.28. MH Provider (MHP). 17
2.29. MH Technician (MHT). 18
2.30. MH Case Manager (MHCM). 19
2.31. MH Office Manager (4A). 19
2.32. Disaster Mental Health (DMH) Team Chief. 19
2.33. Integrated Operational Support (IOS) MH Personnel. 20

Chapter 3—MENTAL HEALTH ADMINISTRATIVE PROCEDURES 22

3.1. Peer Review. 22
3.2. MH Record Management. 22
3.3. Circumstances Necessitating Network Referral Considerations. 22
3.4. After-Hours Command Consultation. 24
3.5. MH Flight Safety. 24
3.6. Self-Inspection Program. 24
3.7. DoW Suicide Event Report (DoDSER). 25

3.8. MH Training Requirements. 25

Chapter 4—OCCUPATIONAL MENTAL HEALTH PROCEDURES 26

4.1. Occupational Medicine Requirements. 26
4.2. Delivery of Care. 26
4.3. Self-Initiated MH Referral Process. 26
4.4. Targeted Care. 26
4.5. Military MH Readiness and Standards. 27
4.6. Unsuiting or Unfitting Conditions. 29
4.7. Special Circumstances. 29
4.8. Management of At-Risk Tracking List (ARTL) Personnel Procedures. 29
4.9. ART Meeting Procedures. 30
4.10. TTM procedures. 30
4.11. Personnel Reliability Assurance Program (PRAP). 30
4.12. MH Readiness and Occupational Health Evaluations and Screenings. 31
4.13. Special Duty Evaluations and Medical Clearances. 31
4.14. Commander-Directed Evaluation (CDE). 31
4.15. Administrative Separations. 33
4.16. Exceptional Family Member Program (EFMP) Evaluations. 34
4.17. Family Member Relocation Clearances (FMRCs). 34
4.18. Sanity Boards. 34
4.19. Expert Witness and Consultation. 34

Chapter 5—MH READINESS PROGRAM 35

5.1. MH Readiness Program (MHRP). 35
5.2. Pre-Event Phase. 35
5.3. Event Phase. 36
5.4. Post-Event Phase. 39
5.5. Installation PRT. 39
5.6. MH Exercise Utilization. 40

Chapter 6—MH OPERATIONAL CLINICAL SERVICES. 41

6.1. Operational Clinical Services (OCS). 41
6.2. Integrated Operational Support (IOS) MH Operations. 41
6.3. Prohibited MH Services Outside of an MTF. 42

6.4. IOS MH Patient and Personnel Safety. 43
6.5. MH OCS in Contingency Environments. 43
Chapter 7—MENTAL HEALTH AND THE LAW 45
7.1. Limited Privilege Suicide Prevention (LPSP). 45
7.2. Confidentiality. 45
7.3. Military Rule of Evidence 513. 46
7.4. Limits of Confidentiality/Informed Consent. 47
Attachment 1—GLOSSARY OF REFERENCES AND SUPPORTING INFORMATION 48

Chapter 1

MENTAL HEALTH OVERVIEW

1.1. Overview. This policy establishes the MH framework and the integration with DAF readiness and occupational health related requirements. It aims to ensure the psychological well-being of SMs and their access to evidence-based MH programs which directly support medically ready SMs and generate ready medics. This policy emphasizes prevention and accessible services across the continuum of care, while delineating roles and responsibilities throughout various levels of command, healthcare delivery teams, and installation support partners.

1.2. Mission. The mission of DAF MH services is to support the readiness of SMs who are eligible for available MH services provided by DoW and DAF MH personnel. Medical Groups (MDGs) support the development of military medical personnel, as well as the wartime skillset and the evaluation and treatment of all SMs IAW DoDI 6000.19, *Military Medical Treatment* DHA-IPM 26-002, *Management of Digital Psychological Assessments Facility Support of Medical Readiness Skills of Health Care Providers.* MH personnel advance the operational readiness and psychological fitness of the warfighter in garrison and contingency environments, safeguarding the health of the force and mission effectiveness. The operational medical support consists of proactive, accessible, and comprehensive MH services across the continuum of care, including prevention, early intervention, assessment, treatment, and consultation. As subject matter experts (SMEs), these professionals address occupational health requirements unique to SMs, fostering SM and unit resilience essential for overcoming challenges, maintaining peak performance, and achieving DAF objectives.

1.3. Vision. To empower all Airmen and Guardians with the psychological fitness and readiness to excel in any mission – anytime, anywhere.

1.4. Scope. To enhance SMs' psychological fitness for global operations by offering comprehensive MH services both in garrison and during contingency operations. This includes proactive prevention through tailored education and training, rapid early intervention to minimize impact on readiness, evidence-based assessment and treatment, dedicated readiness and reintegration support, and expert consultation with leadership. Data-driven program improvements ensure overall effectiveness, while providers execute fitness-for-duty determinations that adhere to medical standards and balance safety risks with mission requirements.

Chapter 2

ROLES AND RESPONSIBILITIES

2.1. Air Force Surgeon General (AF/SG).

2.1.1. Maintains management responsibility for MH readiness. Overseas policy development and implementation, supports personnel and resource requirements, directs strategic planning, and directs AFMEDCOM to implement DAF MH readiness policies.

2.1.2. Manages readiness policies, programs, and processes with AFRC/SG and National Guard Bureau (NGB)/SG.

2.1.3. Appoints Air Force (AF) and United States Space Force (USSF) Director of Psychological Health (DPH) in accordance with (IAW) DoDI 6490.09. **(T-0)**

2.1.4. Is responsible for quality of care in operational setting IAW DoWI 6025.13, *Medical Quality Assurance in the Military Health System*. **(T-0)**

2.2. Air Force Medical Command (AFMEDCOM) Commander.

2.2.1. Provides required personnel and resources to the AFMEDCOM MH Branch to support the MH mission across DAF.

2.2.2. Directs AFMEDCOM to implement DAF MH readiness policies.

2.2.3. Directs readiness requirements to operate in a complex, contested environment.

2.3. USAF Judge Advocate General (AF/JA). Provides legal opinions, instructions, guidance, and assistance regarding DAF MH readiness policies.

2.4. Air Force Director of Psychological Health (AF/DPH).

2.4.1. Establishes the enterprise strategic framework for DAF psychological health, institutionalizing an operational readiness model anchored in psychological safety, mutual respect, and the foundational standard for all Airmen and Guardians to operationalize “being a good human” through ethical leadership, teamwork, and personal performance optimization IAW DoDI 6490.09. **(T-0)**

2.4.2. Manages the development and coordination, distribution, and effective utilization of training materials pertaining to MH for DAF.

2.4.3. Ensures timely support of requests for information, briefings, and other requirements related to MH readiness.

2.4.4. Represents all MH Career Field Managers (CFMs, 4C, 42P, 42S, 44P, 46P, 46YP) in capability development, particularly in development of future force elements/unit type codes.

2.4.5. Coordinates activities of AF/SG MH CFMs as they relate to MH readiness.

2.4.6. Solicits input for strategic planning for DAF MH readiness from AF/SG MH CFMs, Medical Readiness Command (MRC) DPHs, Major Command (MAJCOM) DPHs, MAJCOM Functional Managers (MFMs), and MH Flight leaders.

2.4.7. Chairs and directs, with 4C0X1 CFM, the Mental Health Advisory Board (MHAB), the strategic body responsible for monitoring and reporting on DAF MH readiness.

2.4.8. Chairs and directs MH Strategic Initiative Group and advises principal leaders on the enterprise's strategic direction. This includes coordinating resource requests and providing operational updates to ensure mission alignment.

2.4.9. Serves as the AF MH representative to the HQ DAF Community Action Board (CAB) and Community Action Team (CAT) as outlined in DAFI 90-5001, *Integrated Resilience,* voting AF representative to the DoD Psychological Health and Readiness Council IAW DoDI 6490.09, and DAF representative to the MH Clinical Community IAW DHA-PI 6000.04. **(T-0)**

2.4.10. Monitors and reports on the availability, accessibility, quality, and effectiveness of the continuum of MH services available to SMs and their family members.

2.4.11. Collaborates with DPHs across the DAF, to include USSF and ARC, to provide guidance, share best practices, and support operational needs as appropriate.

2.4.12. Collaborates and provides support to the AF/SG Integrated Operational Support (IOS) branch as well as the True North+ Program Management Office.

2.5. USSF/DPH.

2.5.1. Collaborates with AF DPH and USSF S1/S1Q on matters regarding Guardian MH policy, programming and initiatives IAW DoDI 6490.09. **(T-0)**

2.5.2. Establishes strategic vision and USSF-wide framework for psychological health, advocating for a service culture prioritizing operational readiness achieved through a foundation of good order and discipline, mutual respect, leadership, followership, a team approach, and cultivating an internal drive to optimize personal performance for the benefit of the individual, team, unit, and service. **(T-0)**

2.5.3. Serves as USSF Psychological Health advisor to the Holistic Health Approach (HHA) and primary advisor to Guardian Resilience Team (GRT) Licensed Mental Health Providers (LMHP) on all aspects of Integrated Operational Support (IOS), DAF MH readiness factors, and Guardian warfighter performance optimization.

2.5.4. Advises AF/DPH on appropriate training and utilization of MH personnel supporting Guardians.

2.5.5. Advises USSF on appropriate utilization of MH personnel to include specific specialties integrated at the unit, GRT, and Field Command (FLDCOM) levels for MH readiness and warfighter performance optimization purposes.

2.5.6. Serves as a key stakeholder on the MHAB, advising MH leaders on MH needs of Guardians and USSF mission requirements, and as the USSF MH representative to DAF CAB and CAT.

2.5.7. Communicates MH specialty care concerns and needs received from USSF to AF DPH, servicing MAJCOM as applicable, AFMEDCOM and DHA Network (DHN) MH Lead.

2.5.8. Collaborates with AF DPH, AFMEDCOM MH Branch Chief servicing MAJCOM DPH as applicable, and Medical Readiness Command (MRC) DPH as appropriate on matters related to MH readiness of Guardians and AD MH providers (MHPs) assigned directly into USSF line units.

2.5.9. Primary advisor on MH readiness standards for Guardians in consultation and coordination with the USSF Chief of Aerospace Medicine (SGP).

2.6. AFMEDCOM MH Chief.

2.6.1. Supports AF and USSF DPH in the development and execution of policy and programming requirements.

2.6.2. Oversees and advises on the policy implementation, management and improvement of DAF MH readiness specific to the readiness of MH personnel and deployable medical teams.

2.6.3. Serves as the primary Functional Area Manager (FAM) for all MH enterprise functions and prioritizes inspection requirements IAW 90-302, *The Inspection System of the Department of the Air Force*. **(T-1)**

2.6.4. Supports MH enterprise leaders to ensure effective MH readiness, including occupational medicine related duties specific to DAF mission requirements (e.g. special duty evaluations, fitness for duty related occupational health activities, etc.).

2.6.5. Collaborates with DHA MH Clinical Community and other AFMEDCOM Divisions (e.g. IOS, Aerospace Medicine, etc.) to ensure enterprise-wide, comprehensive MH service delivery.

2.6.6. Supports the development, coordination, and distribution of evidence-based MH readiness related training materials across the DAF.

2.7. AF/SG MH CFMs/Consultants.

2.7.1. Will ensure a comprehensive approach to the utilization and health of DAF MH assets across six career fields: MH Technician (MHT, 4C), Psychologist (42P), Social Work (42S), Psychiatrist (44P), Psychiatric Nurse Practitioner (46YP), and MH Nurse (46P) IAW AFI44-104, *Military and Civilian Consultant Program and Medical Enlisted Career Field Manager Program*. **(T-1)**

2.7.2. Assists AF/DPH in coordinating efforts with all MH CFMs/Consultants ensuring synchronization of training and utilization of MH assets across all MH product lines and programs.

2.7.3. Leverages Comprehensive Medical Readiness Program (CMRP) and Medical Contingency Response Plan (MCRP) to ensure medical readiness of MH AFSCs.

2.8. MRC DPH.

2.8.1. Supports MRC command enabling functions ensuring the health and wellness of DAFmedics.

2.8.2. Provides MH readiness and operations consultation and oversight to MRC commanders, military installation medical components, MAJCOM/DPHs, and 4C0X1 MFMs.

2.8.3. Supports MRC leadership regarding the readiness posture of MH assets across the command through Medical Readiness Decision Support System (MRDSS) compliance, deployment operations, and all other individual readiness requirements and training.

2.8.4. Serves as a liaison for DAF MH programs such as Family Advocacy Program (FAP) and IOS.

2.8.5. Serves as MH liaison with associated DHA Network (DHN), coordinating clinical MH operations and advising leaders of and mitigating impacts on medical readiness of SMs.

2.8.6. Ensures communication and support procedures with and between installation MH flight commanders and applicable ARC DPHs to provide guidance, share best practices, and resolve emerging issues.

2.8.7. Coordinates with installation MH flight commanders to provide support to geographically separated units (GSUs) and IOS programs as appropriate.

2.8.8. Collaborates with appropriate AFMEDCOM MH Branch Chief and MAJCOM DPH on Staff Assistance Visits (SAVs).

2.8.9. Utilizes the Management Internal Control Toolkit (MICT) to review applicable MDG subordinate units' compliance at least twice annually to provide recommendations to improve unit effectiveness.

2.8.10. Liaises with MAJCOM DPH and DHN MH lead to monitor MH operations impacting readiness requirements.

2.8.11. Advises applicable installation MH flight commander on the management and coordination of conflicts of interest cases and cases of active-duty MH personnel with identified occupational impairment.

2.9. MAJCOM/SG.

2.9.1. Implements and ensures compliance with DAF MH policies at the MAJCOM level.

2.9.2. Appoints MAJCOM DPH to serve as a subject matter expert on MH related topics within the command.

2.10. MAJCOM DPH/Combatant Command DPH (CCMD/DPH).

2.10.1. MH readiness subject matter expert to MAJCOM or CCMD commander. MAJCOM DPH will support USSF FLDCOMs until specific positions are formally allocated.

2.10.2. Serves as the MAJCOM program manager overseeing IOS programs executing across the MAJCOM.

2.10.3. Collaborates with MAJCOM/FLDCOM CAT and CAB to provide expert consultation and leadership in the development, implementation, and evaluation of community MH programs, ensuring alignment with current evidence-based practices and industry standards.

2.10.4. Fosters a culture of well-being by promoting healthy behaviors, facilitating access to relevant resources, and encouraging informed decision-making throughout the organization and within the community. **(T-1)**

2.10.5. Develops and executes MAJCOM/FLDCOM-specific MH initiatives to address requirements in coordination with the MRC DPH and AFMEDCOM MH Branch Chief.

2.10.6. Coordinates with AF and USSF DPH, AFMEDCOM MH Branch Chief, MRC DPH on issues pertaining to MH readiness. Develops MH initiatives targeting MAJCOM/FLDCOM-specific issues and requirements.

2.10.7. Collaborates with appropriate MRC/MAJCOM DPH and 4C MFM in the requests for SAVs, in-person or virtually, and utilizes the MICT to determine MDG compliance at least quarterly and provides recommendations to improve unit effectiveness.

2.10.8. Liaises with MRC and DHN MH leads to monitor MH operations impacting readiness requirements.

2.11. 4C0X1 MAJCOM Functional Manager (MFM) or equivalent.

2.11.1. Encouraged to conduct site visits to review and advise on training requirements for their commands, implement deployment readiness training platforms and elevate concerns to the 4C0X1 CFM IAW AFH-1, *Airman*. **(T-1)**

2.11.2. Ensures utilization of 4C0X1 personnel as defined in the 4C0X1 Career Field Education & Training Plan (CFETP). **(T-1)**

2.11.3. Coordinates with applicable AFMEDCOM MRC DPH and/or DHN MH lead on issues pertaining to MH readiness or training that are relevant or impact 4C0X1 duties, training, and readiness requirements.

2.12. Mental Health Advisory Board (MHAB).

2.12.1. Chaired by AF DPH and 4C0X1 CFM and convenes monthly.

2.12.2. Provides strategic guidance and programming recommendations for the MH enterprise through AFMDECOM leadership and respective Corps leadership.

2.12.3. Composed of: AF DPH, USSF DPH, 4C0X1 CFM, AFMEDCOM MH staff, MAJCOM MH staff, MRC MH staff, MH AFSC CFMs/Consultants (4C, 42P, 42S, 44P, 46P, 46YP), MH AFSC Training Directors and ARC DPHs.

2.13. Installation Commander.

2.13.1. Designates an individual to serve as the installation DPH IAW DoDI 6490.09. **(T-0)**

2.13.2. Promotes collaboration among installation organizations to foster healthy and resilient communities where the baseline expectation is to actively support fellow Airmen and Guardians IAW DAFI 90-5001. **(T-1)**

2.13.3. Ensures Command Post notification to MH of any all-hazard incidents as part of base notification protocol for Disaster Mental Health (DMH) team response IAW DoDI 6200.03, *Public Health Emergency Management (PHEM) within the Department of Defense*. **(T-0)**

2.13.4. Secures formal agreements for provision of personnel and/or resources to include but not limited to joint service partners, ARC, or non-military organizations required for program effectiveness. **(T-0)**

2.13.5. Ensures GSUs have access to installation DMH team based on geographic proximity and unit needs. **(T-0)**

2.13.6. Ensures unit commanders consult with the DMH Team Chief regarding pre-exposure preparation training, long and short-term interventions, unit assessment of functioning and other prevention initiatives for all-hazard incidents or operational stress reactions as defined in DoDI6490.05. **(T-0)**

2.14. NGB DPH will refer to ANG policy and directives.

2.15. Air Force Reserve Installation DPH (AFR IDPH).

2.15.1. Are permanent General Schedule (Title 5) employees who report directly to their respective installation commander. In absence of a full-time installation commander, the DPH reports to the deputy installation commander. **(T-2)**

2.15.2. Serves as the AFR installation commander MH SME, informs relevant AFR leaders of MH trends through AFR wing MH plan, utilization reports and existing CAB/CAT functions. **(T-1)**

2.15.3. When co-located on an AD installation, serves in non-clinical and clinical roles as an affiliate provider with local installation MTF clinical privileges, DMH team member as appropriate, and adheres to occupational health training requirements. **(T-1)**

2.15.4. Complies with all requirements for operational clinical services IAW DoWI, 6025.13, including documenting as appropriate in the electronic health record (EHR). IAW DAFMAN 10-211, when a SM's presenting issues fall within the clinical domain (based on clinical acuity or meeting criteria for a Diagnostic & Statistical Manual diagnosis), care must be documented in the SM's EHR. **(T-1)**

2.15.5. Is available on-site during duty hours and UTAs. When unavailable, DPH shall provide contact information to installation law enforcement and community MH support. **(T-2)**

2.15.6. Supports installation Sexual Assault Prevention and Response (SAPR) IAW this instruction section 2.23.8. **(T-1)**

2.15.7. Attends installation CAT and CAB IAW AFI 90-5001. **(T-1)**

2.15.8. Coordinates with Security Forces Squadron (SFS) annually to conduct security/threat assessments IAW UFC 4-510-01 and DAFMAN 10-211. The AFR wing commander may consider necessary actions according to the assessment recommendations pertaining to DPH operational environment to ensure the safety of all personnel. **(T-1)**

2.16. Installation Staff Judge Advocate (SJA). Ensures DAF JA office is available to MH personnel in need of legal consultation on relevant DAF policy interpretation and military readiness issues.

2.17. MDG Commander and Reserve Medical Group Commander or equivalent.

2.17.1. Ensures MH personnel receive dedicated time to effectively complete readiness training requirements, regardless of their assigned unit and duties. Confirms training compliance is monitored in the MRDSS IAW DAFI 41-106, *Medical Readiness Program*. **(T-1)**

2.17.2. Provides a safe physical environment for MH personnel and patients within the MTF by implementing safety requirements to include duress systems, controlled access to MH staff offices, and other measures to support safety IAW DoD Unified Facilities Criteria 4-510-01 Appendix A Incorporating Change 4,15 July 2025, and DoD Space Planning Criteria Chapter 318: Mental Health Ambulatory Care Services. **(T-0)**

2.17.3. Appoints the DMH Team Chief IAW DoDI 6200.03. **(T-0)**

2.18. Chief of Medical Staff (SGH).

2.18.1. Shares responsibility with Chief of Aerospace Medicine (SGP) for all provider training to ensure all AD patients are evaluated for duty/mobility restrictions at each visit. **(T-2)**

2.18.2. Notifies applicable HQ ARC equivalents SG offices for death by suicide or suicide attempts involving ARC SMs for Line of Duty determination. MHPs provide input on AF 348 to the Investigating Officer IAW DAFI 36-2910, *Department of the Air Force Guidance Memorandum (DAFGM) to DAFI 36-2910, Line of Duty (LOD) Determination, Medical Continuation (MEDCON), and Incapacitation (INCAP) Pay*. **(T-1)**

2.18.3. Along with the SGP, will ensure providers utilize current Initial Review In Lieu Of (IRILO) narrative summary (NARSUM) templates for MH/traumatic brain injury conditions IAW AFI 48-133. **(T-2)**

2.18.4. Provides oversight and necessary support for clinical case management services for MH patients should a full-time equivalent case manager not be available. **(T-1)**

2.19. Chief of Aerospace Medicine (SGP).

2.19.1. Supports all Airmen Medical Readiness Optimization (AMRO) meetings related to any DAF SM that requires a profile, to include following a suicide attempt, to determine the readiness disposition of the SM. **(T-1)**

2.19.2. Ensures MH personnel are adhering to profiling practices and duty limitation standards IAW AFI 48-133, *Duty Limiting Conditions*. **(T-1)**

2.19.3. Validates medical waivers for deployment-limiting conditions related to MH conditions in collaboration with Base Operational Medical Cell, (BOMC) IAW DAFI 48-122, *Deployment Health* and considering impact to Space Force Generation (SPAFORGEN) employed-in-place operations. **(T-1)**

2.20. Squadron Commander Responsible for MH Flight.

2.20.1. Assigns a privileged MHP to serve as the MH flight commander to ensure the effective management of service specific programs and readiness requirements. **(T-1)**

2.20.2. Ensures 4C0X1 utilization is prioritized to meet the full capabilities as outlined in the 4C0X1 CEFTP. **(T-1)**

2.20.3. Will allocate time for full-spectrum MH readiness:

2.20.3.1. To ensure mission success, leaders must allocate non-direct patient care time for MH personnel to complete all readiness related requirements. **(T-2)**

2.20.3.2. Non-direct patient care duties are mission essential including, but not limited to, DMH duties, Postvention Response Team (PRT) duties, command consultations, occupational MH readiness and service specific requirements outlined in **Chapter 4** of this instruction, installation functions (e.g. CAT), unit and community engagement, and all required training for garrison and contingency environments. **(T-2)**

2.20.3.3. Leaders must protect their providers and technicians through direct engagement and advocacy by engaging with medical chain of command, SGP, SGH, MH flight commander, MAJCOM DPH, DHN to develop mitigation plans when provider wellness, readiness requirement, or care demands exceed resources. This balance is essential to the psychological readiness of the installation and operational effectiveness of the MH team. **(T-2)**

2.21. MH Flight Commander (or equivalent).

2.21.1. Provides operational and administrative oversight of the MH flight.

2.21.2. Ensures implementation of all DoW, DHA, and DAF policy and guidance. Establishes local policies and standard operating procedures, as necessary, to define procedures that may not be specified in higher policy regarding DAF readiness requirements. **(T-3)**

2.21.3. Consults with the MDG leadership on the adequacy of installation MH flight staffing and identified conflicts among DAF programs, clinical business, and MH readiness requirements in coordination with the MRC DPH, DHN MH Lead, MAJCOM DPH and MAJCOM MFM or equivalent within AFMEDCOM.

2.21.4. Ensures initial completion and periodic review of onboarding, annual and sustainment training for MH flight personnel. Ensures MH personnel review relevant onboarding topics and materials prior to engaging in direct patient care as it pertains to occupational health and readiness. Training plans and resources are available on AFMS Knowledge Exchange. **(T-2)**

2.21.5. Responsible for completion and documentation of MH readiness training requirements, including but not limited to all MCRP and CMRP requirements for all AD MH personnel IAW DAFI 41-106. **(T-1)**

2.21.6. Advises the squadron commander on AFSC disqualification, retention, or downgrade of MH flight personnel IAW the Personnel Services Delivery (PSD) Guide Classification Enlisted AFSC Disqualification and PSD Guide Classification Officer AFSC Disqualification. **(T-1)**

2.21.7. Ensures implementation of the MH Readiness Program (MHRP) postvention offering support and resources to clinical staff experiencing the psychosocial impact of a patient death by suicide. Complies with health care provider resiliency and support service as indicated in DHA Procedural Instruction (DHA-PI) 6025.17, *Healthcare Resolutions, Disclosure, Clinical Conflict Management and Healthcare Provider (HCP) Resiliency and Support in the Military Health System (MHS).* Similar resources are available for other adverse events as needed to support medical personnel well-being. Resources are available on AFMS Knowledge Exchange. **(T-1)**

2.21.8. Ensures an MH SME is available to the Exceptional Family Member Program (EFMP) IAW AFI 40-701, *Medical Support to Family Member Relocation and Exceptional Family Member Program (EFMP).* **(T-1)**

2.21.9. Adheres to the defined/published hours of operations for outpatient care within the MDG facility for emergency MH evaluations both during and after duty hours. After hours evaluations will only be conducted in a military medical treatment facility \(MTF) with an emergency room or urgent care for the safety of the MH flight, patients and MDG staff. Military MTFs without an emergency department or urgent care capabilities will send after-hours MH evaluations to a local civilian medical facility emergency room. **(T-1)**

2.21.10. Ensures a MHP supports the local AMRO Board IAW DAFMAN 48-108, *Physical Evaluation Board Liaison Officer (PEBLO) Functions: Pre-Disability Evaluations System (DES) & Medical Evaluation Board (MEB) Processing,* and DAFMAN 48-123, *Medical Examinations and Standards*. **(T-1)**

2.22. MH Flight Chief.

2.22.1. Manages MH flight operations as the lead enlisted advisor to the MH flight commander.

2.22.2. Serves as the local 4C Functional Manager providing oversight, mentorship, and support to all 4Cs on the installation. Ensures MHTs are utilized to full capabilities defined in the 4C0X1 CFETP and DHA-PI 6490.12. **(T-1)**

2.22.3. Ensures MHTs are allotted time to complete patient care duties IAW DHA-AI 6490.12, and AFSC requirements in coordination with the MH flight commander. (**T-1)**

2.22.4. Advises the MH flight commander on AFSC disqualification, retention, or downgrade of 4Cs within the MH flight IAW the PSD Guide Classification Enlisted AFSC Disqualification. **(T-1)**

2.22.5. Oversees MH flight training for all MH personnel, to include but not limited to MCRP and CMRP requirements. Provides oversight for training and certification management (obtaining and maintaining) for Certified Alcohol and Drug Counselors (CADC) within the MH Flight. **(T-1)**

2.22.6. Enforces DAFMAN 36-2689, *Training Program* and ensures all technician training is appropriately documented in designated training records. Oversees evaluation, feedback, and recognition processes and monitors all upgrade training for personnel in MH. **(T-1)**

2.22.7. Manages all resource allocations and monitors/requests updates to the AD unit manning document ensuring proper manning to support local installation missions.

2.23. Installation DPH (IDPH).

2.23.1. Serves as the principal consultant for MH on behalf of the installation commander or comparable activity commander, as determined by the Secretary of the Air Force IAW DoDI 6490.09. **(T-0)**

2.23.2. Ensures installation key stakeholders (i.e. commanders, First Sergeants, etc.) are educated on available installation and community MH resources. Coordinates efforts in the execution of MH education and training for SMs across the installation.

2.23.3. Provides relevant installation MH updates to the Integrated Primary Prevention Workforce (IPPW) and installation commander to guide development and execution of support and resources available to the community IAW DAFI 90-5001. (**T-1)**

2.23.4. Serves as the MH representative to the installation CAB and CAT meetings. ARC IDPH will attend and serve as ARC MH representative when co-located on a DAF installation. Will provide MH consultation and relevant data to ensure both appropriate utilization of MH resources and community needs are met. **(T-1)**

2.23.5. Consults with MDG MH flight and IOS MH personnel on issues related to their respective lines of effort to include MH readiness, warfighter psychological performance optimization, and occupational health requirements.

2.23.6. Coordinates efforts with installation Integrated Prevention Response Director (IPRD) and all helping agencies, military and non-military, in the execution of MH education and training for SMs across the installation and GSUs.

2.23.7. Collaborates with key stakeholders to implement the MHRP focused on mitigating the impact of cumulative stress and occupational dysfunction among SMs across the installation. Stakeholders consist of all MH personnel (e.g. flight, IOS, etc.)

2.23.8. Serves as a MH consultant for the installation SAPR monthly Case Management Group (CMG), quarterly CMG (QCMG), and serves as the MHP who fills the role of the informed representative to help facilitate the exchange of information regarding MH needs and victim safety, IAW DoDI 6495.02 Volume 1 *Adult Sexual Assault Prevention and Response Program Procedures* and DAFI 90-6001, *Sexual Assault Prevention and Response Program.* **(T-1)**

2.23.8.1. IDPHs role in the quarterly CMG is to discuss response system coordination and access to service trends separate from individual case management oversight that occurs in the CMG.

2.23.8.2. Collaborates with primary clinicians supporting those engaged with SAPR services ensuring comprehensive trauma-informed support provided across agencies. **(T-0)**

2.24. MH Flight Medical Director.

2.24.1. Is a psychiatrist, prescribing psychologist, or psychiatric nurse practitioner designated by the MH flight commander. If a prescribing provider is not available within the MH flight, this position can be delegated to another prescribing provider or PCM IAW with the AFMS Flight Path. **(T-1)**

2.24.2. Provides all requirements as outlined in the AFMS Flight Path. **(T-1)**

2.24.3. Advises MDG/CC on EFMP cases IAW AFI 40-701. **(T-1)**

2.25. MH Element Chief.

2.25.1. Ensures effective clinic operations and support to MH element personnel.

2.25.2. Monitors and reports MH data, which may include, but is not limited to, training compliance, exercise after action reports, DHA activities (e.g., relevant HEDIS metrics, access to care), and readiness requirements as stipulated by the MDG/CC, MAJCOM/SG, MRC/CC, or AFMEDCOM leadership.

2.25.3. Maintains oversight of all occupational medicine related requirements pertaining to MH flight operations, including but not limited to those outlined in: DAFI 36-3211, *Military Separations*, DAFMAN 48-108*,* DAFMAN 48-123, AFI 48-133, *Duty Limiting Conditions,* DoDI 6490.07, *Deployment-Limiting Medical Conditions for Service Members*, and DoWI 6490.04, *Mental Health Evaluations of Service Members*. **(T-1)**

2.26. Alcohol and Drug Abuse Prevention and Treatment (ADAPT). Element Chief supports MH flight IAW DAFI 44-121, *Alcohol and Drug Abuse Prevention and Treatment Program*. **(T-1)**

2.27. FAP Manager Element Chief. Supports the MH flight IAW DoDI 6400.01 and DAFI 40-301, *Family Advocacy Program*. **(T-0)**

2.28. MH Provider (MHP).

2.28.1. Adheres to all applicable DoW, DHA, and DAF policy and guidance referenced in this instruction. **(T-0)**

2.28.2. Determines fitness and suitability for duty, mobility, security clearance level, and deployment, deployed in place at each encounter, or Employed-in-Place for USSF; refer to AMRO as appropriate IAW DAFMAN 48-108. **(T-1)**

2.28.3. Consults with commanders on MH readiness IAW DoDI 6490.08. **(T-0)**

2.28.4. Completes MH IRILO/MEB MH NARSUMs when indicated, IAW DAFMAN 48-108. **(T-1)**

2.28.5. Adheres to Personnel Reliability Assurance Program (PRAP) regulations, notification requirements, and track individuals assigned to Personnel Reliability Program (PRP) & Arming and Use of Force (AUoF) IAW DoDM 5210.42_DAFMAN 13-501, *Nuclear Weapons Personnel Reliability Program (PRP)* and DAFI 31-117 *Arming and Use of Force*. **(T-0)**

2.28.6. Complies with IOS policy IAW DAFMAN 10-211, *Integrated Operations Support*, and AFMAN 48-149, *Flight and Operational Medical Program*, when providing Operational Clinical Services (OCS). **(T-1)**

2.28.7. Refers individuals to EFMP IAW AFI 40-701. **(T-1)**

2.28.8. Manages at-risk patients and participates in At-Risk Tracking (ART) activities IAW DHA-AI 6025.06, ensuring a thorough assessment of occupational health requirements. **(T-0)**

2.28.9. AD MHPs will complete requirements IAW the CMRP checklist maintained on the AFMS Knowledge Exchange, and the local readiness office readiness training schedule IAW DoDI 6490.05, *Maintenance of Psychological Health in Military Operations*. **(T-0)** Relevant MCRP training will be completed IAW DAFI 41-106 and individual team training requirements. **(T-0)**

2.28.10. Screens and/or assesses for special duty assignments as required, including but not limited to: AUoF, PRP, Presidential Support Program (PSP), Military Training Instructor (MTI), Military Training Leader (MTL), and security clearance evaluations. **(T-1)**

2.28.11. Identifies concerns that may impair, distract, or otherwise interfere with an individual's ability to safely carry out military duties are reported to the appropriate authority IAW DoDI6490.08. **(T-0)** I n the event these concerns fall outside the scope of practice or expertise for the involved provider(s), MH personnel will consult and/or refer to appropriate SMEs to resolve any risk to the military mission.

2.28.12. Utilizes Air Force Enlisted Classification Directory (AFECD) and Air Force Officer Classification Directory (AFOCD) when assessing SMs suitability for entry and retention in specific career fields and report any concerns impacting their qualifications and fitness for duty, or risk to the military mission IAW DoDI 6490.08. **(T-0)**

2.28.13. Performs Commander-Directed MH Evaluations (CDEs) IAW DoWI 6490.04. **(T-0)**

2.28.14. Psychiatrists and psychologists are qualified to conduct military sanity boards within the confines of licensure and boundaries of competence. **(T-0)**

2.28.15. May provide scientific expert testimony in military courts-martial when they possess the appropriate qualifications. Any request to serve as an expert witness in litigation for a party other than the United States must be processed IAW AFI-51-301. **(T-1)**

2.28.16. If engaged in care with a sexual assault victim, will support the IDPH in their efforts to serve the SAPR CMG and High-Risk Response Team as appropriate IAW DoDI 6495.02, Volume 1, Enclosure 9. **(T-0)**

2.28.17. Will report every incident of sexual assault involving SMs to the applicable program office, specifically SAPR or FAP offices IAW DoDIs 6310.09, 6495.02, Vol 1&2, DAFI 90-6001, DAFI 40-301. (**T-0)**

2.29. MH Technician (MHT).

2.29.1. Adheres to all applicable DoW, DAF, and DHA instructions referenced in this instruction including but not limited to DoDI 6490.08, DoDM 5210.42, DAFMAN 10-211, AFMAN 48-149, AFMAN 13-501, DHA-AI 6490.12, and the AF CADC Handbook. **(T-0)**

2.29.2. Performs patient care related duties IAW training and skill level under oversight of a privileged MHP or CADC and DHA-PI 6490.12. **(T-1)**

2.29.2.1. Are provider extenders within the MH flight and will be involved in clinical assessments with clinical supervision.

2.29.2.2. Able to provide clinical care IAW their clinical skills and training with clinical supervision in the MH flight as outlined by the CFETP. **(T-1)**

2.29.3. Assesses patient's fitness and suitability for duty, mobility, and deployment at each patient encounter. **(T-1)**

2.29.4. Refers individuals to the EFMP IAW DAFI 36- 2110. **(T-1)**

2.29.5. Completes required pre-deployment, CMRP, MCRP, upgrade training requirements and any other required training to maintain CADC certification. **(T-1)**

2.30. MH Case Manager (MHCM).

2.30.1. Oversees the management of patients identified as high-risk and/or high-cost diagnoses and treatment. Coordinates and manages at-risk patients including performing clinical management and administrative requirements.

2.30.2. Attends required multidisciplinary team meetings. **(T-1)**

2.30.3. Provides indirect patient care under the supervision of a privileged MHP, in support of treatment plans with patient outcomes aligned with best practices based on the unique needs of each patient. **(T-1)**

2.30.4. Coordinates with treating facilities, providers and command teams, on and off installation, on treatment requirement and transition needs. **(T-1)**

2.30.5. If an MHCM is not present the MH flight, will coordinate with MDG/CC and SGH to utilize available resources to support continuity of care for MTF beneficiaries. **(T-3)**

2.30.6. Understands the occupational MH and administrative requirements associated with managing members on mobility/duty restricting profiles and/or SMs placed on the At-Risk Tracking List (ARTL).

2.31. MH Office Manager (4A).

2.31.1. Provides direct support to MH flight administrative operations to include but not limited to assisting patients and clinical staff to conduct clinical mission requirements.

2.31.2. Assists MH flight leadership with identifying and managing clinic capacity and demand by utilizing historical workload data, developing and maintaining provider templates and managing appointment templates and schedules.

2.31.3. Coordinates patient referrals with the Referral Management Center (RMC).

2.31.4. Coordinates with appropriate clinical and support functions to ensure all required documents are available for each patient encounter.

2.31.5. Performs clinical front desk responsibilities to include but not limited to checking in patients using various systems, verifying DEERS information, assisting patients with required clinic documents prior to appointment, and assisting with scheduling follow up appointments.

2.31.6. Performs end-of-day processing at the end of each clinic day IAW local MDG guidance.

2.31.7. Performs other patient administrative functions consistent with the 4A skillset as appropriate to the clinic setting.

2.32. Disaster Mental Health (DMH) Team Chief.

2.32.1. Is an independently licensed MHP IAW DoDI 6200.03 (**T-1)**; alternate team chief can be either an officer, senior noncommissioned officer, or noncommissioned officer from the MH flight trained to execute the DMH team mission.

2.32.2. Integrates DMH team preparedness and response plans with the MCRP. Facilitates relevant support agreements with local MH support agencies to ensure coordinated delivery of services following an all-hazards incident. **(T-2)** See Terms section of this instruction for DoW definition of “all-hazards incident.”.

2.32.3. Coordinates with the public health emergency officer and medical emergency manager to integrate with public health emergency preparedness and response plans**. (T-2)**

2.32.4. Collaborates with installation SMEs from MH, spiritual support, family support, and primary prevention offices (e.g., IPPW, Military and Family Readiness Center) to direct and deliver DMH services to the population assigned to the installation impacted by all-hazards incidents IAW DoDI 6200.03. **(T-0)** All AD MH personnel support the DMH team during an all-hazards response for an effective response to the community need. **(T-1)**

2.32.5. Supports installation emergency management working groups to develop appropriate scenarios for DMH response IAW DoDI 6055.17, *Department of Defense Emergency Management Program* and DAFI 10-2501, *Emergency Management Program*. **(T-0)**

2.32.6. Oversees operationally relevant scenario-based MH exercises IAW installation/wing/delta exercise program and participates in installation emergency management exercises IAW the installation/wing/delta and MDG exercise program. **(T-2)**

2.32.7. Ensures DMH training of key stakeholders and services are completed IAW DoDI6200.03, to include annual training for installation and unit leadership. **(T-0)**

2.32.8. Integrates deliberate work-rest cycles and recovery time into disaster response operations to mitigate secondary traumatic stress and ensure force preservation among MH personnel. **(T-3)**

2.32.9. Collaborates with key stakeholders to implement the MHRP to mitigate the impact of cumulative stress among SMs across the installation. **(T-3)**

2.32.10. Collaborates with ARC unit or regional DPH and will develop and utilize resources under their control to support DMH and maintain partnerships with the regular component where proximity allows.

2.32.11. Ensures only evidence-informed and evidence-based approaches are applied during exercise and real-world incidents. **(T-1)**

2.32.12. Supports PRT activities IAW DTM 25-004, *DoD Suicide Postvention Response System* and this instruction. **(T-0)**

2.33. Integrated Operational Support (IOS) MH Personnel.

2.33.1. Adhere to all applicable DoW, DHA, and DAF policy and guidance referenced in this instruction. **(T-0)**

2.33.2. Adhere to MH requirements associated with the respective positions outlined in DAFI 44-172, 2.28 or 2.29 and maintain currency and competency for their readiness requirements. **(T-1)**

2.33.3. Privileged IOS MH personnel will provide clinical MH treatment IAW DHA MH System of Care, DHA Suicide Risk Care Pathway, DHA Targeted Care and inTransition guidance. **(T-1)**

2.33.4. Attend each clinical consultation meeting within the MDG, including but not limited to MH at-risk meetings to support their respective patients being managed and relevant Aerospace Medicine meetings (e.g. AMRO, FOMWG, etc.) as appropriate for patients they are managing. IOS MH personnel are expected to attend MDG-required professional staff meetings (e.g. ProStaff) and relevant career field CMRP/MCRP MDG training requirements and exercises IAW DAFMAN 10-211 and DAFI 41-106. All readiness training requirements and completion of training will be documented in MRDSS or designated training record system according to MDG policy. **(T-3)**

2.33.5. Collaborates with MH flight leaders to ensure a comprehensive approach to community support utilizing clinical and non-clinical resources. **(T-3)**

2.33.6. Assists with DMH operations and responses with approval from the DMH Team Chief and respective IOS commanders as appropriate for events that occur outside their currently assigned unit. **(T-3)**

2.33.7. At a minimum, use the AF/SG MH onboarding toolkit to develop foundational MH operations knowledge. As directed by the MTF privileging authority, identified preceptor and IOS provider will develop a well-defined, time-limited, well-documented plan to complete all Focused Professional Practice Evaluation (FPPE) requirements upon initial privileging. **(T-1)**

2.33.8. MHTs possess and maintain a current CADC certification or equivalent certification. Will not execute ADAPT requirements outside of the MTF. Will work under the clinical supervision of a privileged licensed MHP to maintain CADC competency and currency IAW DAFI 44-121. **(T-1)**

2.33.9. Civilian MHTs without a CADC working in an IOS setting will be limited to outreach and prevention activities. Activities in a clinical paraprofessional role will be dictated by the position description or performance of work statement. **(T-1)**

Chapter 3

MENTAL HEALTH ADMINISTRATIVE PROCEDURES

3.1. Peer Review.

3.1.1. MH personnel follow peer review guidance IAW DHA-PM 6025.13 utilizing service specific templates/approved forms found on the AFMS Knowledge Exchange until DHA publishes a tri-service/standardized MH peer review form and process. **(T-0)**

3.1.2. In addition to DHA peer review guidance, 100% of at-risk cases seen in the last 30 days per provider per month will be reviewed to ensure occupational and readiness requirements are met and communicated to the SM's chain of command. **(T-1)**

3.2. MH Record Management.

3.2.1. All MHPs/MHTs will adhere to clinical documentation processes to ensure necessary readiness requirements are captured at each encounter. In contingencies where there is no electronic communication, documentation will be completed on SF600. Hard copy clinical information will be scanned/uploaded into the EHR once capabilities are available. **(T-1)**

3.2.2. Sanity Board Reports.

3.2.2.1. Long and short sanity board reports will be filed in a physical MH record unless otherwise authorized by the convening authority or, after referral of charges, by the military judge. **(T-1)**

3.2.2.2. These reports will be labeled "Do Not Release Unless Approved by Proper Legal Authority after Consultation with Staff Judge Advocate (SJA) or "for Medical Reasons."

3.2.2.3. These reports are not captured in a clinical encounter and are not documented in the EHR. Sanity board reports are not considered a medical encounter but an evaluation for legal determination. **(T-1)**

3.2.3. Psychological assessments will adhere to DHA-IPM 26-002, *Management of Digital Psychological Assessments*. **(T-1)**

3.2.4. Records are maintained and dispositioned in accordance with the Air Force Records Disposition Schedule, which is located in the Air Force Records Information Management System. Physical MH records will follow record retirement procedures IAW AFI 33-322. **(T-1)**

3.3. Circumstances Necessitating Network Referral Considerations.

3.3.1. SMs referred off base for outpatient clinical services are contacted to assess clinical needs, military occupational readiness, and fitness for duty, at a minimum of every 6 months or more frequently if clinic resources can support. SMs should be reminded of the requirement to report any new medical conditions or change in medical status to the appropriate medical provider at the time of onset IAW DoDI 6025.19. If indicated, the clinic will contact SMs if operational medical readiness determinations are required, (e.g. to determine deployment medical clearance). **(T-3)**

3.3.2. MHPs will notify the SMs' commander if any of the exigent circumstances outlined in DoDI 6490.08 exist. Commanders will be informed of any concerns regarding increased risk, whether a CDE is recommended or warranted, advised of any duty limiting conditions or restrictions exist, or profiles that may affect mobility or PCS capabilities. **(T-0)**

3.3.3. Transfer and termination of MH services will adhere to DoWI 6490.10 and DHA-AI 6490.02. **(T-0)**

3.3.4. MHPs will ensure a "warm handoff" when transferring care either within the DoW, to Veterans Affairs, or to a civilian provider. MHPs will ensure eligible SMs are enrolled in the in Transition program and educate SMs on how to access that resource to ensure care continuity. **(T-0)**

3.3.5. MHPs will promptly prioritize and complete case closure/termination documentation via the last follow-up note which captures clinical reasoning for closure, readiness status, occupational related limitations, and any referral recommendations.

3.3.6. No show/missed appointment procedures will adhere to DoDI 6490.08 and DHA-AI6025.06. **(T-0)**

3.3.7. Circumstances may arise where SMs in need of MH care have a conflict of interest within the MH flight, such as when they are members of that flight, command that flight, or have personal relationships with all providers within that flight.

3.3.7.1. MHPs and MHTs will adhere to all applicable DoW, DHA, and DAF policy and guidance for delivering MH services to include, but not limited to DoDI 6490.08, DoWI6490.10, DHA-AI 6490.01, DHA-AI 6490.02, and DHA-AI 6025.06. **(T-0)**

3.3.7.2. MH personnel will adhere to their professional specialty's Code of Ethics during any conflict of interest. Element leadership will resolve identified conflicts, elevating them through the chain of command or through the SGH as necessary. **(T-1)**

3.3.7.3. In the event there is a conflict of interest in the management of a patient (e.g. MH flight personnel), referrals to the civilian network for outpatient MH treatment may be authorized.

3.3.7.4. To mitigate conflicts of interest, MH personnel and other potential patients with a verified conflict of interest will not receive clinical MH treatment from within their assigned flight. The MH flight commander, in consultation with the SGH, will establish a local policy to address these cases. This policy will designate that MH care of MH personnel is managed by the PCM and treated by an off-base provider. For cases requiring occupational health or readiness determinations (e.g., profiles, CDEs, or fitness-for-duty evaluations), SGH will serve as the primary medical authority, coordinating with an appropriate MHP from a different installation or via the MRC/MAJCOM DPH to ensure an impartial evaluation is completed. **(T-3)**

3.3.7.5. In most circumstances, without an identifiable reason to deviate, the MH flight commander will make arrangements to ensure MH staff receiving MH services are managed by appropriate providers/case managers outside of the MH flight (e.g. PCM. medical case manager) with the SGH acting as consultant. **(T-3)**

3.4. After-Hours Command Consultation.

3.4.1. After-hours MH support is restricted to consultation for unit leadership, law enforcement, and medical personnel managing emergencies accessed via the installation Command Post. All direct patient emergencies must be routed to local emergency services. Non-emergent clinical requests will be deferred to the appropriate IOS or MDG asset for follow-up during the next duty day. **(T-1**)

3.4.2. No direct patient assessments or treatment is authorized via the after-hours MH consultant. **(T-1)**

3.4.3. For the purpose of this instruction "after hours" includes MTF's not open for patient care for any reason (e.g., closed for training day). **(T-1)**

3.4.4. Installations with an MDG Emergency Department (ED) or Urgent Care Center (UCC) will provide after-hours MH assessments. At installations without an ED or UCC, MH related emergencies will be managed using community medical resources IAW MDG-established protocols. **T-1)**

3.5. MH Flight Safety.

3.5.1. The MH flight commander will ensure there is adequate staffing and leadership awareness when appointments extend past MDG duty hours as dictated by local policy.

3.5.2. To ensure the safety of MH personnel and patients, MH safety protocols will be tested monthly in coordination with Security Forces Squadron (SFS). Exercises will be conducted and documented at least annually, in coordination with the SFS and installation Inspector General (IG) office. **(T-2).**

3.5.3. MH personnel do not provide transportation of any MH patients. The primary mode of transportation for emergency MH situations is Emergency Medical Services (EMS). Where acute safety concerns are not present, the unit may transport, as appropriate. **(T-1)**

3.5.4. For SMs suspected of experiencing a psychiatric emergency outside of the MTF, the following resources will be utilized: 911, 988, base emergency services, or local/host nation emergency services. **(T-1)** If there are no acute safety concerns, the SM may be voluntarily escorted by another SM to the MH clinic for evaluation during regular duty hours.

3.6. Self-Inspection Program.

3.6.1. The Commander's Inspection Program purpose is to assess and improve readiness, efficiency, discipline, compliance, and enhance mission effectiveness. The self-inspection program for both garrison and expeditionary units is executed IAW DAFI 90-302, *The Inspection System of the Department of the Air Force*. **(T-1)**

3.6.2. The MH flight commander and flight chief (garrison and deployed/contingency) should adhere to MDG MICT business rules as it pertains to self-inspection process.

3.6.3. Expeditionary MH personnel operate under unique mission environments. The deployed MH flight must inform the deployed medical commander of any identified areas of risk to personnel and the mission.

3.7. DoW Suicide Event Report (DoDSER).

3.7.1. MH personnel will collaborate with IPPW providing clinically relevant information as appropriate for completion of the DoDSER IAW DoDI 6490.16, *Defense Suicide Prevention Program*. **(T-0)**

3.7.2. DoDSER is completed for SMs during periods of military service. For ARC members who are not activated, the MH portion of the DoDSER will be completed by the relevant Reserve/Guard DPH. DoDSER completion is not required for government civilians, contractors, or military dependents. **(T-0)**

3.7.3. DoDSER is utilized to garner information pertaining to suicide attempts and deaths by suicide. Suicidal ideation is not tracked by DAF entities, given the high prevalence of suicidal ideation and its limited predictive value for suicide attempts. **(T-1)**

3.8. MH Training Requirements.

3.8.1. Upon hiring, all MH personnel will review and complete required readiness and occupational health training requirements in addition to DHA required training for all new MH personnel.

3.8.2. Training requirements below are captured to codify occupational health requirements unique to military MH. All training materials and resources located on the AFMS Knowledge Exchange will be trained on an annual basis. **(T-1)**

3.8.2.1. Management of at-risk patients (e.g., suicide and homicide) and ARTL procedures.

3.8.2.2. CDEs and administration separation process and procedures.

3.8.2.3. Special duty status evaluations: PRAP (PRP & AUoF) and security clearances.

3.8.2.4. Duty Limiting Condition (DLC), profile management, AMRO and disability evaluation system policy and procedures.

3.8.2.5. Sanity Boards. There is no formal training requirement to conduct a sanity board. Psychiatrists and psychologists are authorized to conduct these non-clinical evaluations IAW Rules for Court Martial (R.C.M.) 706 based on their academic degree and license. **(T-0)** Consultation and training is available upon request by DAF forensic MH community. Training resources are available on the AFMS Knowledge Exchange.

Chapter 4

OCCUPATIONAL MENTAL HEALTH PROCEDURES

4.1. Occupational Medicine Requirements.

4.1.1. MH personnel must evaluate, validate, and determine SM's fitness for duty, while considering the specific demands of military occupations, potential impact of MH conditions on occupational duties, and rendering professional judgment and recommendations that prioritize both SM well-being and mission readiness and effectiveness.

4.1.2. MHPs must complete annual training on various occupational MH tasks and requirements conducted by local flight SMEs. Refer to AFMS Knowledge Exchange for training materials and tools. **(T-3)**

4.2. Delivery of Care.

4.2.1. Across all environments (in garrison, deployed, and all OCS locations), MH will prioritize patient and staff safety, patient-centered care, and optimal clinical outcomes.

4.2.2. During contingency operations where facilities, supplies, or personnel are constrained, MH personnel will adapt care delivery to meet these standards to the maximum extent feasible following leadership consultation.

4.3. Self-Initiated MH Referral Process.

4.3.1. IAW DoWI 6490.04, DoD-DTM 23-005 and DAFI 90-5001, SMs may request a MH evaluation referral to the MTF from their commanding officer or any supervisor in the grade of E-6 or above. SMs may request this referral at any time and in any environment. SMs are not required to provide a reason or basis to their chain of command to request or receive this referral. **(T-0)**

4.3.2. MHPs will assess SM's medical readiness/fitness for duty as soon as possible and, when practical, provide necessary care as clinically indicated following communication with the commander or supervisor consistent with DoDI 6490.08 and DTM 23-005. Medical readiness for duty is assessed with specific consideration for MH condition, risk of harm to self or others, symptom severity, prognosis for return to duty and risk of decompensation, aggravation or further injury if participation in occupational activities continues.

4.3.3. Evaluations may be completed via any approved medium (e.g. in-person, virtual, etc.)

4.3.4. Self-initiated MH evaluations are held to the same confidentiality rules as voluntary care, meaning no feedback to commander unless a DoDI 6490.08 exigent circumstance is present. (**T-1)**

4.4. Targeted Care.

4.4.1. Targeted Care is a standard MH workflow which ensures a structured and efficient approach to patient care with the goals of matching individuals to the best clinical and/or non-clinical resource for their presenting concern and increasing access to resources, reducing wait times, and enhancing readiness.

4.4.2. Targeted Care allows same day and walk-in support without requiring a referral. Through the vectoring process, Targeted Care is designed to identify whether additional assessment is clinically indicated. SM's needs are aligned with the most appropriate medical or non-medical counseling resource. MHPs operating in OCS locations support Targeted Care processes for the SM population they support. **(T-1)**

4.4.3. If a victim of sexual assault or intimate partner violence calls or walks into the facility/clinic, they should receive timely access to care to assess for safety concerns and to develop an initial plan for further assessment and treatment. Victim services will be IAW with DAFI 40-301 and DAFI 90-6001. **(T-1)**

4.4.4. Utilization of groups is an appropriate, evidence-based approach to delivering MH psychoeducation and manualized psychotherapy. **(T-1)**

4.5. Military MH Readiness and Standards.

4.5.1. The primary mission of DAF MH is to optimize the psychological readiness and worldwide qualification of every SM. When psychological health impacts mission performance, MH professionals will deliver evidence-based treatment protocols focused on rapid recovery and a return to full functioning and duty status.

4.5.2. When duty or mobility limitations are identified during the course of treatment, MHPs and appliable MHTs will communicate with SM's leadership using the Aeromedical Services Information Management System (ASIMS), the Airman or Guardian Availability Management (AGAM) module. Documentation will capture the restrictions and the clinical rationale in the EHR, and the MHP will verbally inform SM of the limitations. Information provided to SM's commander will be limited to "need to know" based on risk and medical recommendations of duty and mobility limitations. Inconsistent communication hinders standardization, effective care, and commander's risk mitigation efforts. AF Form 469, *Duty Limiting Condition Report,* is the DAF standard for communicating medically related duty or mobility restrictions IAW DoDI6490.07 *&* AFI 48-133. For sister-service personnel, providers will document and communicate readiness limitations to commanders utilizing the service-specific profile system or pre-identified notification process. Commanders will use the profile recommendations to make final duty or mobility decisions. **(T-1)**

4.5.3. MHPs will adhere to the following, providing minimum necessary disclosure:

4.5.3.1. Utilize profiles to communicate duty and/or mobility restriction recommendations to commanders based on their clinical judgement and recommended profiling guidelines.

4.5.3.2. Avoid overly restrictive profile limitations that hinder a commander's ability to keep a member engaged in an occupational function within the unit and that can exacerbate isolation and stigma.

4.5.3.3. Advise commanders on how to best support a SM's treatment plan with an emphasis to return SMs to full duty as soon as clinically appropriate.

4.5.3.4. IAW DoDI 6490.07 and AFI 48-133, continually evaluate and document in the EHR at every patient encounter, SMs ability to:

4.5.3.4.1. PCS, temporary duty, and/or deploy.

4.5.3.4.2. Perform specific job-related duties.

4.5.3.4.3. Maintain suitability for access to classified information as applicable to the member.

4.5.3.4.4. Meet medical retention standards at every clinical contact. **(T-1)**

4.5.3.5. When considering any SM's mobility or duty restrictions, reference the Medical Standards Directory (MSD) Section Q as a tool to assist in determining clinical and functional limitations of the SM. For sister-service personnel, consult the service-specific medical standards and collaborate with the SM's command regarding recommendation requirements and procedures.

4.5.3.6. At every encounter, must provide a comprehensive review of MH conditions that may cause limitations to deploy, perform occupational duties, or meet retention standards. If any limitations are clinically identified, initiate a profile via the AF Form 469 in the ASIMS/AGAM module (or service-specific system) with appropriate recommended mobility/duty restrictions. Any modification or extension of limitations should be accompanied by appropriate clinical justification. **(T-1)**

4.5.3.7. Consider SMs with an MH disorder with occupational dysfunction for a profile based on impairment that negatively interferes with their ability to fulfill their occupational duties/expectations.

4.5.3.7.1. Conditions expected to resolve within 12 months will be assigned an Assignment Availability Code 31 on the AF Form 469 (or service-specific requirement). **(T-1)**

4.5.3.7.2. For SMs with an MH disorder that is likely to interfere with ability to fulfill their duties or long-term worldwide availability exceeding 365 days, refer SM to the AMRO Board for determination and disposition. SMs may be referred to AMRO for review sooner than the 365-day mark if they are not responding to treatment or have a poor prognosis. **(T-1)**

4.5.3.8. When a SM's MH symptoms impair occupational performance, are undergoing active treatment, have been stable for fewer than three months, result in residual symptoms impairing duty performance, or pose a substantial risk for deterioration, encouraged to consult with another clinical MHP to assess that SM's fitness for duty in a contingency environment IAW DoDI6490.07. **(T-0)**

4.5.3.9. Adhere to guidance outlined in DoDM 5210.42_DAFMAN 13-501, and guidance in DAFI 31-117, which outlines AUoF requirements to ensure SMs' ability to safely perform armed duties when MHPs are delivering services across the Targeted Care continuum. **(T-1)**

4.5.3.10. Communicate any decisions to initiate, modify, or discontinue a duty or mobility restricting profile with patient. **(T-1)**

4.5.3.11. Assess SMs actively engaged in MH clinical treatment identified for PCS or deployment to verify members' ability to fulfill their duties at their gaining PCS or deployment location IAW AFI 48-133. **(T-1)**

4.6. Unsuiting or Unfitting Conditions. SMs with an unfitting or unsuiting condition that will exceed or is reasonably anticipated to exceed 365 cumulative calendar days should be referred to the AMRO Board IAW AFI 48-133 & DAFMAN 48-108. For MH conditions/diagnoses warranting consideration of administrative separation refer to **paragraph 4.8.4** of this instruction and DAFI 36-3211. **(T-1)**

4.7. Special Circumstances. MHPs will notify and advise the Competent Medical Authority (CMA)/Flight Medicine Clinic regarding recommended limitations or restrictions associated with members on special duty status IAW DoDM 5210.42_DAFMAN 13-501. The MDG flight medicine clinic or CMA will review SMs status prior to resuming flight, controlling, or other special duties IAW DAFMAN 48-123. **(T-1)**

4.8. Management of At-Risk Tracking List (ARTL) Personnel Procedures.

4.8.1. All DAF MHPs will follow DHA guidance regarding SMs at intermediate or high risk for dangerousness (i.e. increased risk of harm to self or others, or harm to mission) IAW the VA/DoD Clinical Practice Guidelines for Assessment and Management of Patients at Risk for Suicide and DHA-AI 6025.06. Placement on, removal from, duration on the ARTL are clinical decisions based on evidence-based and evidence-informed practices, in addition to a comprehensive risk assessment, clinical data, collateral information, patient self-assessment, and professional judgement. **(T-1)**

4.8.2. DAF MHPs will follow guidance and include proper screening and education on lethal means safety as a vital part of safety planning. Lethal means counseling includes asking about the presence of all lethal means of suicide and working with patients and their support network to safely store and/or limit access to those means. MH personnel will educate individuals with suicide risk and their support network, with patient consent and when appropriate, about safe firearm storage and access, as well as the appropriate storage of alcoholic beverages, prescription drugs, over-the-counter medications, and poisons that may be available where they reside. **(T-1)**

4.8.3. In addition to the procedures outlined in DHA-AI 6025.06, DAF MH personnel will accomplish the following tasks associated with the ARTL: post-hospitalization discharge planning, ART meeting, and Treatment Team Meetings (TTMs). All ARTL scheduled appointments will be labeled as ARTL in the EHR reason for visit section. **(T-1)**

4.8.3.1. For post-discharge planning from higher levels of care, to include inpatient psychiatric care, residential care, partial hospitalization, intensive outpatient, or substance abuse rehabilitation, a MHP will assess military personnel for fitness for duty and safety NLT the next duty day after discharge. Clinical discretion will drive whether fitness for duty and safety is assessed in-person versus virtually. **(T-1)**

4.8.3.2. MH personnel will ensure appropriate notifications are completed to the applicable program monitors for SMs on PRP/AUoF/PSP/Flight status, etc or who possess a Top Secret or higher security clearance IAW DoDI 6025.18 *Health Insurance Portability and Accountability Act (HIPAA) Privacy Rule Compliance in DoD Health Care Programs*, DoDM 5210.42_DAFMAN 13-501, DoDI 6490.08, DAFI 31-117, and DAFMAN 48-123. Notifications must be documented in the EHR. **(T-0)**

4.8.4. MDG members who meet criteria for inclusion on the ARTL will be tracked in the same way as all other patients, unless there is a bona fide conflict of interest that has been identified at which point the SGH will oversee referral for appropriate care. **(T-1)**

4.9. ART Meeting Procedures.

4.9.1. The ART meeting will occur weekly to review patients on the ARTL and will be documented in the EHR. Main purpose of the meeting is to verify current risk level, hospitalization status, discharge follow up appointments, progress with the treatment plan, case management decisions, and whether the current at-risk status is still warranted. **(T-1)**

4.9.2. Clinical discussions in the meeting will have a multidisciplinary focus with all elements and specialties across the MH flight (e.g. ADAPT, FAP, PCMH, IOS) involved to ensure continuity of care, risk management across elements, and the fitness for duty restrictions of patients are discussed and captured. Documentation of treatment decisions and clinical rationale will be captured in the EHR as clinically appropriate and serve as notification to PCMs for continuity of care. **(T-1)**

4.9.3. SMs engaged in multiple elements within the MH flight should be staffed during ARTL meetings, regardless of their level of suicide risk, to ensure thorough case coordination and safety planning. **(T-1)**

4.10. TTM procedures.

4.10.1. TTM for SMs managed through the ARTL process will be conducted within 7 calendar days of addition on the ARTL or discharge from a higher level of care. **(T-1)**

4.10.2. At a minimum, TTM will include MHP, SM, commander/First Sergeant or designee. Primary Care Manager (PCM) or flight surgeon are invited and encouraged to attend the TTM but are not required attendees of the meeting. If PCMs/flight surgeons are not present during TTM, they are notified of TTM outcomes for continuity of care and aeromedical disposition via the EHR. SMs may request to have other individuals who are active in supporting their care or safety plan attend TTMs. **(T-1)**

4.10.3. The treatment team will be informed on the ARTL process, relevant treatment/safety planning, duty/mobility limitations, and possible readiness implications (e.g. DLC, duty retention, or other medical disposition). **(T-1)**

4.10.4. Details regarding the TTM will be captured in the EHR. TTM template is available on the AFMS Knowledge Exchange.

4.11. Personnel Reliability Assurance Program (PRAP).

4.11.1. For SMs being certified, recertified, or otherwise evaluated for PRP, potential suitability factors shall be identified based on impairment to their reliability, judgement, or suitability to perform PRP duties and adjudicated by a CMA to determine if immediate notification to the certifying official or their designated monitor is necessary IAW DoDM 5210.42_DAFMAN 13-501. **(T-0)**

4.11.2. For FAP related possible suitability factors, refer to DoDM5210.42_DAFMAN 13-501 and DAFI 40-301 regarding FAP system of records and adjudication for PRP personnel. **(T-0)**

4.11.3. For SMs on AUoF status, any condition(s) impairing an individual's reliability to safely perform his/her duties will be reported immediately to their commander and/or designated command representative IAW DAFI 31-117. **(T-1)**

4.12. MH Readiness and Occupational Health Evaluations and Screenings.

4.12.1. MH readiness and occupational health screenings or evaluations are utilized to assist in the evaluation of safety, fitness, reliability, judgement and/or suitability for military duties or to determine eligibility for specific special duty positions or requirements.

4.12.2. MH readiness and occupational health appointments will be documented in the EHR utilizing REDNS as an appointment type which clearly differentiates this workload from clinical treatment to adequately capture this readiness and occupational MH requirement. **(T-1)**

4.12.3. If an MH disorder is identified, the provider will code that as a secondary diagnosis for this specific visit only. Reference the latest version of the DHA *Mental Health Coding and Documentation Handbook* for the most current and accurate coding practices.

4.13. Special Duty Evaluations and Medical Clearances.

4.13.1. BOMC is responsible for all medical clearance workflows IAW AFMAN 48-149, and DAFI 48-122. This includes review of the SMs' medical record, identification of medical or MH conditions requiring a waiver or further consultation, and clearance disposition, as appropriate/indicated. **(T-1)**

4.13.2. If medical documentation related to care received in a MH flight is unclear, BOMC will consult with the appropriate MHP and/or the MDG MH element chief or MH medical director for clarification and assistance with specific questions and provide disposition recommendations.

4.13.3. All MH personnel will document any identified duty, mobility, and deployment limitations in ASIMS/AGAM and the medical record, at every encounter.

4.13.4. MH personnel may be required to assist in clearances for special duties including security clearances. Signed consent for the release of confidential information is not required to disclose fitness for duty information to command, IAW DoDI 6490.08. **(T-1)** Special Duty evaluations will be conducted IAW the specific evaluation requirements.

4.13.5. IOS MH personnel may conduct readiness and occupational MH related evaluations and clearances in collaboration with the MH element chief. Relevant evaluations/clearances will occur within the MTF and documented in the EHR.

4.13.6. Current information and guidance regarding how to complete MH related occupational health evaluations and clearances is located on the AFMS Knowledge Exchange.

4.14. Commander-Directed Evaluation (CDE).

4.14.1. CDEs will be conducted IAW DoWI 6490.04 and this instruction and have the same status as any other military order. A CDE checklist, training materials, and relevant templates are available on the AFMS Knowledge Exchange. Utilization of these tools is recommended to ensure all CDE tasks are completed and meet standards.

4.14.2. CDEs do not apply to voluntary self-referrals or any special duty or occupational classification evaluations. **(T-0)**

4.14.3. CDEs in garrison will be conducted within the MTF's MH flight during duty hours. Installations with ED or UCC will provide after-hours emergency CDEs. At installations without an ED or UCC, emergency CDEs will be managed using community medical resources IAW MDG-established protocols to assess for safety concerns. **(T-1)**

4.14.4. Routine/non-emergent CDE requests are appropriate should the commander or supervisor identify concerns including, but not limited to, fitness for duty, occupational requirements, significant changes in performance, or behavioral changes that may be attributable to possible MH diagnoses and or mental status changes. **(T-0)**

4.14.4.1. For purposes of a routine CDE, a commander is defined as any commissioned officer who exercises command authority over a SM. **(T-0)**

4.14.4.2. For purposes of a routine CDE, a supervisor is defined as a commissioned officer within or out of a SM's official chain of command, or civilian employee in a grade level comparable to a commissioned officer, who exercises supervisory authority over the SM owing to the SM's current or temporary duty assignment or other circumstances of the SM's duty assignment; and is authorized due to the impracticality of involving a commanding officer in the chain of command to direct a CDE. **(T-0)**

4.14.5. Emergency CDEs are appropriate should the commander or supervisor have concerns about potential or imminent danger to self or others. **(T-0)** For emergency CDEs regarding enlisted SMs, a senior enlisted SM may be designated by the commander or supervisor to refer. A First Sergeant is an appropriate referral source for emergency CDEs in DAF. **(T-0)**

4.14.6. MHPs will assist the commander or supervisor with initiation of the CDE, defining the reason for CDE request, and addressing specific duty-related and/or safety concerns. **(T-1)**

4.14.7. For the purposes of an outpatient CDE, a MHP is defined as a psychiatrist, clinical psychologist, licensed clinical social worker, psychiatric nurse practitioner, or other licensed clinical MHP who is credentialed and privileged by DHA or MILDEPs IAW IAW DoWI6490.04. For situations involving an inpatient CDE referral, only a MHP with admitting privileges may evaluate and admit the SM. **(T-0)**

4.14.8. MHP will obtain the required CDE information from the requesting commander or supervisor and/or medical record IAW DoWI 6490.04 prior to the appointment. **(T-0)**

4.14.9. MHP will ensure SMs understand they are being evaluated by the order of the commander or supervisor IAW DoWI 6490.04, and informed consent paperwork has been reviewed. **(T-0)**

4.14.10. For emergency and routine CDEs, MHP will provide verbal feedback to the requesting commander/commander representative on the same day of the evaluation. **(T-1)**

4.14.10.1. For cases where a routine CDE evaluation occurs over multiple days, MHP will provide relevant updates to the referring commander consistent with DoWI 6490.04 and DoDI 6490.08. **(T-1)**

4.14.10.2. For emergency and routine CDEs, MHP will provide written feedback to the referring commander or commander representative no later than three business days after the evaluation is completed, and the memorandum will be uploaded in the EHR. **(T-1)**

4.14.10.3. The EHR encounter will include all relevant assessment information and recommendations including timeline of command notification and feedback. **(T-1)**

4.15. Administrative Separations.

4.15.1. When an MH evaluation and subsequent diagnosis results in a recommendation for administrative separation of a SM, the primary MHP must determine whether special processing is required IAW DoDI 1332.14, DoDI 1332.30, and DAFI 36-3211. **(T-0)** MHPs will reference the AFMS Knowledge Exchange for additional procedural guidance.

4.15.2. IAW DAFI 36-3211, an MH professional is permitted to formally recommend administrative separation to the SM's commander. **(T-1)**

4.15.3. Administrative separation evaluation and required supporting documents, administrative separation recommendation memorandum and if applicable, the special processing endorsement memorandum will be captured in the EHR. **(T-1)**

4.15.4. IAW DAFI 36-3211, SMs diagnosed with a personality disorder or other MH disorder not constituting a physical disability, are entitled to special processing for discharge if they experienced one or more of the following during their military service: (**T-0)**

4.15.4.1. Have history of or are currently serving in an imminent danger pay area. **(T-0)**

4.15.4.2. Have filed an unrestricted report of sexual assault. **(T-0)**

4.15.4.3. Have disclosed they are victim of a sex-related, intimate partner violence or abuse offense. **(T-0)**

4.15.5. Cases that meet criteria for special processing are required to: (**T-0)**

4.15.5.1. Be corroborated by a peer or higher-level MH professional, who has generally similar training, privileges or clinical experience. If a peer or higher-level MH professional is not available locally, provider may seek disposition consultation with MAJCOM DPH or MRC DPH. **(T-0)**

4.15.5.2. Be endorsed by AF/SG, AFRC/SG, or NGB/SG or designee. **(T-0)**

4.15.5.3. Address PTSD and other mental illness co-morbidity. NOTE: Separation will not be initiated if there is a service-related PTSD diagnosis, unless properly screened for eligibility into the Disability Evaluation System process and when applicable, found the SM fit and/or a returned to duty determination has been rendered. **(T-0)**

4.15.6. For specific entry level separation processes information, refer to DAFI 36-3211 and AFMS Knowledge Exchange. For sister service evaluations, refer to service guidance.

4.15.6.1. When separating initial entry SMs (Total Force officer training, BMT, METC), who have exceeded entry level separation timelines, DAF must demonstrate the member had an opportunity to overcome their deficiencies unless waived by the member, IAW DAFI 36-3211. 4.15.6.2. In some cases, MHPs may need to submit additional documentation indicating a return to training would only further aggravate the condition or that the member's condition is not likely to improve enough to allow for a return to training. **(T-1)**

4.16. Exceptional Family Member Program (EFMP) Evaluations. MH personnel will provide referrals to the Special Needs Coordinator for conditions that meet criteria for EFMP enrollment IAW DAFI 36-2110, *Total Force Assignments*, and AFI 40-701. **(T-1)**

4.17. Family Member Relocation Clearances (FMRCs). MHPs advise on relevant outbound or inbound facility determination inquiries. **(T-1)**

4.18. Sanity Boards.

4.18.1. As requested by legal counsel and IAW R.C.M. 706, MHPs may provide consultation regarding MH related questions that are raised formally during legal proceedings. Psychiatrists and psychologists are qualified to conduct military sanity boards within the scope of their licensure and training. Examiner expenses for sanity boards are not DHP funded. **(T-0)**

4.18.2. IOS MH personnel may participate or support sanity boards evaluations in collaboration with MH flight and unit leadership approval.

4.18.3. While MHPs may assist with this formal legal request, it should be noted that as DHP funded personnel, regular patient care duties take precedence.

4.19. Expert Witness and Consultation.

4.19.1. All MHPs may testify as expert consultants in military courts-martial with approval from the local chain of command.

4.19.2. AFI 51-301 governs consultation and testimony in other settings and requires approval by the appropriate legal office.

4.19.3. Permission to consult and/or testify as an expert witness in non-military justice matters will be submitted through the appropriate base-level/servicing SJA to the AF Civil Law and Litigation Directorate. **(T-1)**

Chapter 5

MH READINESS PROGRAM

5.1. MH Readiness Program (MHRP).

5.1.1. The MHRP is a framework designed to enhance the readiness of SMs and MH personnel in garrison and contingency operations. The primary mission is to improve mission execution in complex, contested environments by preventing and managing combat and operational stress through increased psychological resilience and skill-building both before and after exposure to stressful or wartime events.

5.1.2. MH flight commander and flight chief will integrate and synchronize the MHRP framework. Implementing MHRP requires all MH personnel both in garrison and contingency operations to comply with the Combat and Operational Stress Control principles outlined in DoDI 6490.05. **(T-0)** Framework model and additional resources are available on AFMS Knowledge Exchange to use as a tool to ease implementation and increase collaboration across installation helping agencies.

5.1.3. Executing MHRP duties may differ depending on the unit assigned and the available resources at the location. During contingency or deployment operations, the majority of MHRP will be provided by the MH personnel assigned to that specific location or area of responsibility.

5.1.4. Implementation will include multiple agencies across the installation/area of responsibility in addition to MH flight resources throughout all MHRP phases. Other agencies include, but are not limited to, IPPW, fire department, security forces, emergency services, mortuary affairs, casualty affairs, and various installation helping agencies.

5.1.5. MH flight leaders including DMH team chief and IDPH will collaborate and implement the MHRP by identifying methods of engagement with units across their installation, consulting with unit leaders, IOS MH assets, and installation helping agency members to determine evidence-based and evidence-informed support. **(T-1)**

5.1.6. MHRP framework guides the utilization of installation MH resources in support of enhancing psychological readiness of SMs and units. This framework prioritizes personal responsibility of MH and connects to CJCSI 3405.01, *Chairman's Total Force Fitness Framework* and the DAF's Comprehensive Airman Fitness Program. **(T-1)**

5.1.7. MHRP framework is intended to be broad to allow flexibility in the utilization of resources, whether to support a response to an acute adverse event or large-scale event. MHRP utilizes a phased approach, within the following phases: pre-event, event, and post-event.

5.2. Pre-Event Phase.

5.2.1. The pre-event phase focuses on proactive prevention. MH personnel will train on and deliver evidence-based strategies to sustain psychological fitness, mitigate operational stress, posture members for effective resilience responses, and build combat stabilization skills across the force. **(T-1)**

5.2.2. MH has a critical role in prevention and will follow a population health model focused on proactive prevention utilizing DAF resources to enhance knowledge and skills pertaining to MH.

5.2.3. IDPH, in collaboration with MH personnel, will engage in first-hand surveillance activities IAW DoDI 6490.05. These activities include, but are not limited to, command consultation, identification of at-risk populations, and evaluation or surveillance of units periodically and after exceptionally stressful events. **(T-0)**

5.2.4. All MH personnel will consult with unit leaders on MH concerns impacting unit readiness and engage installation members outside the MTF to provide tailored support and resources.

5.2.5. MH personnel will leverage Pre-Exposure Preparation (PEP) principles to prepare units for exposure reactions. **(T-1)**

5.2.5.1. PEP's objective is to enhance personal readiness and strengthen the mental and emotional fortitude of our personnel. This training equips individuals with the skills to master stress responses, maintain peak performance under pressure, and cultivate the readiness necessary to excel in demanding environments. **(T-0)**

5.2.5.2. MH will provide PEP training to installation stakeholders whose professional responsibilities place them at a higher risk for exposure to potentially traumatic events. MH personnel should coordinate PEP training with leaders of units expecting exposure to potentially traumatic events such as combat exposure or exposure to all-hazards incidents, when both parties agree the training is warranted. **(T-0)**

5.2.5.3. PEP training should be tailored to specific unit requirements and expected exposures and should not duplicate existing training. AD MHPs and MHTs will provide fundamental MH-related psychoeducation that is operationally relevant for SM and unit mission. If a unit has dedicated MH personnel assigned, they will execute this training requirement. **(T-0)**

5.2.6. At any time, commanders may coordinate with MH flight leadership to request AD MH personnel to offer consultation for preventive and education services to unit leaders, EMS, law enforcement agencies, First Sergeants and other helping agencies. **(T-1)**

5.3. Event Phase.

5.3.1. The event phase involves the activation of appropriate helping agencies to meet community needs based on the nature of the activating all-hazards incident or as requested by installation leadership.

5.3.2. The events captured in the term all-hazards incident are not inclusive. Response capabilities are essential for any event on the installation where helping agency resources could improve the health and well-being of those directly and indirectly involved.

5.3.3. DMH is the primary tool, led by the DMH Team Chief or designated alternate, throughout the event phase. DMH is a multidisciplinary resource installation leaders use to assist with the execution of a coordinated comprehensive community response following an all-hazards incident. **(T-1)**

5.3.4. DMH team fosters resilience in those who have been or may be exposed to all-hazards incidents or operational stress environments.

5.3.5. DMH services include education and support for those likely to experience significant or high stress events/experiences and stressors, ongoing psychoeducation, screening, intervention, psychological first aid, traumatic event management, and referrals for clinical services as needed for those exposed.

5.3.6. Installation DMH team membership will include at a minimum:

5.3.6.1. MDG MH provider, who serves as DMH Team Chief. **(T-2)**

5.3.6.2. MH personnel from the MDG and installation IOS MH programs. **(T-2)**

5.3.6.3. Religious Support Team (RST). **(T-2)**

5.3.6.4. IPPW personnel. **(T-2)**

5.3.6.5. Military and Family Readiness Center & Community Readiness Consultant. **(T-2)**

5.3.6.6. Other relevant helping agencies fitting to assist in the response, who may serve ad hoc or as permanent members, including Employee Assistance Program (EAP), SAPR, security forces, civil engineers, master resilience trainers at the discretion of the DMH Team Chief and proposed member's leadership IAW DoDI 6490.05 and DoDI 6200.03. **(T-0)**

5.3.7. DMH Team Chief, upon consultation with impacted unit leadership, will determine the level of mobilization required; this includes, but is not limited to, an "all-hands" response. During all-hazard responses it is expected that all MH personnel will support DMH operations and interruption to normal MH flight operations will occur. MH flight leadership and DMH Team Chief will coordinate with MDG chain of command on expected clinic disruptions and risk mitigation planning. Formal coordination between multiple agencies outside the MDG will occur IAW DoDI 6200.03, DoDI 6200.03, DoDI 6490.05 and DHA-PI 6055.17. **(T-0)**

5.3.8. When MH is unable to support due to a conflict of interest or when MH is impacted by the event, the MH flight will coordinate with their local leadership and MRC and/or MAJCOM DPH and MFM for support from another installation. **(T-2)**

5.3.9. DMH team training:

5.3.9.1. DMH team training will occur at least quarterly for all team members and will focus on realistic all-hazards response education and preparation. All DMH training and exercise activities and scenarios will be conducted as a team IAW DAFI 41-106. **(T-2)**

5.3.9.2. Minimum training topics to occur on an annual basis, include but not limited to PEP, outreach, screening, triage, psychological first aid, education and referral for clinical services for individuals and groups, command consultation, ethical issues, needs assessments and surveillance, moral distress or injury, and burnout and secondary trauma.

5.3.10. Installation DMH utilization:

5.3.10.1. During an active event, MH personnel will not respond or be on-scene and will not engage with individuals exhibiting threatening behaviors. SFS and EMS will be consulted as appropriate for assistance. **(T-1)**

5.3.10.2. All individuals directly involved in all-hazards incident or operational stress event will be provided the opportunity to access DMH services without undue command influence IAW DoDI 6490.05 and DoDI 6200.03. **(T-0)**

5.3.10.3. Participation in DMH services is voluntary, and DMH providers will provide verbal informed consent outlining the voluntary nature of DMH services and limits to confidentiality. **(T-0)**

5.3.10.4. Commanders can request DMH services be provided to their unit, SM's participation remains voluntary.

5.3.10.5. DMH is an educational/consultative intervention and is not formal MH treatment. This difference must be communicated to DMH recipients and leadership when responding to all-hazards incidents. **(T-1)**

5.3.10.6. Following an all-hazards incident, individuals can seek one-on-one consultation with any member of the DMH team IAW DoDI 6200.03. DMH consultations provide SMs education and support and do not provide medical assessment or treatment. Interventions are classified as educational services focused on activating incidents and are not documented in the health record. Members seeking services outside the focus of the activating incident should be referred to applicable resources for formal treatment or support. SMs requiring or requesting clinical MH assessment, will be referred to the MTF for clinical MH assessment. **(T-1)**

5.3.10.7. Government Service civilians and contractors seeking DMH education services should be referred to the EAP or their applicable medical plan. **(T-0) EAP** offers critical resources to civilian personnel affected by all-hazards incidents. Some locations may have additional resources/supports that may be able to augment the team.

5.3.10.8. Individuals serving as first responders or participating in search and rescue activities, including professional personnel (e.g., pathologists, legal, casualty affairs, mortuary personnel, etc.) should have the opportunity to receive DMH services IAW DoDI 6200.03 and DoDI6490.05. **(T-0)**

5.3.10.9. When PRP or AUoF regulations require DMH providers to disclose DMH client information, DMH provider will limit disclosures to information specific to impairment in performance of PRP/AUoF duties. **(T-0)**

5.3.10.9.1. If services elevate to treatment, the provider must comply with notification requirements outlined in DoDM 5210.42_DAFMAN 13-501, DAFI 31-117, AFMAN 48-149, AFI 44-119, *Medical Quality Operations*, & DHA-PM 6025.13, *Medical Quality Assurance and Clinical Quality Management in the Military Health System*. **(T-0)**

5.3.10.9.2. PRAP notifications will be made to the appropriate individuals as designated on the HIPAA or CMA roster IAW applicable regulations. Any required notifications will be performed immediately and must occur before the individual performs PRAP duties. **(T-0)**

5.3.10.9.3. Suitability factors should be identified based on impairment in performance of PRP/AUoF duties. **(T-0)**

5.4. Post-Event Phase.

5.4.1. This phase prioritizes maintaining and supporting personnel psychological readiness immediately after and in the long-term after an event. MH personnel will provide ongoing support through targeted postvention, functional restoration as needed, and the re-establishment of psychological readiness following a traumatic event or contingency operation.

5.4.2. MH personnel who do not have a conflict of interest and who are not at risk for secondary trauma stemming from the incident will provide post-event support through the DMH team following an all-hazards incident. **(T-1)**

5.4.3. Following a death by suicide, MH flight personnel who do not have a conflict of interest and who are not at risk for secondary trauma stemming from the suicide will support the Postvention Response Team (PRT). **(T-1)**

5.4.4. The DMH team may still be active during this phase to provide unit and individual support as necessary. All helping agency resources will be utilized based on their scope and practice capabilities and the needs of the community.

5.4.5. Postvention is an organized response implemented after a suicide death to reduce further suicide risk and promote healing. The three phases of postvention (Stabilize, Grieve, and Grow) are a useful guide to support survivors, while understanding each individual's response will vary. MH personnel will partner with installation helping agencies to ensure policies, programs, practices and support are available to those in need IAW DTM 25-004, DAFI 90-5001 and all associated DAF supplemental guidance. **(T-0)**

5.4.6. Restoration emphasizes SMs personal responsibility and mirrors the proactive nature of the prevention phase, highlighting the importance of continuous self-reflection, help-seeking behavior, and peer support.

5.4.7. Re-establishment focuses on a unit's adjustment recognizing the psychological impact of an incident can extend far beyond the initial response, while proactive interventions can facilitate positive psychological change and enhance long-term well-being. **(T-1)**

5.4.8. MH personnel will collaborate with installation helping agencies to provide psychoeducation fostering resilience, connecting with social support and serve as a consultant to unit leaders for unit events to include but not limited to strategic communication, unit engagement, and memorial services. **(T-1)**

5.4.9. SM's requiring clinical MH services to include, but not limited to, counseling of MH disorders, will be referred to a MH professional. Clinical judgement will be utilized to determine acuity level and care location needs. **(T-0)**

5.5. Installation PRT.

5.5.1. PRT will be utilized following the notification of a SM suspected suicide. PRT can be utilized after a suspected suicide of unit member civilians, contractors, spouses, or dependents. PRT will follow DTM 25-004 and other DAF postvention guidance. **(T-0)**

5.5.2. PRT is a collaborative approach to postvention in support of the suicide impacted unit and command team. The team will not conduct inspections or evaluations. The team will be engaged following a death by suicide for immediate support and will continue to be engaged for the months following as appropriate.

5.5.3. MH personnel will serve in a supporting role of the PRT as MH SME for clinical matters. **(T-1)**

5.5.4. PRT response must also prioritize the care team's psychological welfare. Recognizing patient/colleague suicide can impact health care personnel as "second victims," specific postvention protocols will be deployed within clinical settings to mitigate professional distress, preserve psychological safety, and ensure immediate, non-punitive access to peer and professional counseling and support. **(T-1)**

5.5.5. To preserve patient safety and protect staff well-being, temporarily pause, reduce, or reassign clinical duties for directly impacted personnel following a patient or colleague suicide is recommended. This operational adjustment allows for immediate postvention support, peer debriefing, and a compassionate, structured assessment of a provider's readiness to resume clinical care. **(T-1)**

5.6. MH Exercise Utilization.

5.6.1. Exercise scenarios will be operationally relevant and will evaluate the ability to survive, respond to, recover from, and operate under realistic threat conditions or all-hazards incidents, and/or to meet specific installation or Combatant Commander requirements in line with DAFI 90-302 and DAFI 41-106. **(T-1)**

5.6.2. All installation MH personnel, to include both MDG and IOS MH personnel, will participate in exercise scenarios. **(T-3)**

5.6.3. DMH Team Chief will collaborate with the installation IG and exercise planning committees to develop and identify operationally relevant training scenarios and rules of engagement to test the response capabilities of the DMH team. **(T-3)**

Chapter 6

MH OPERATIONAL CLINICAL SERVICES.

6.1. Operational Clinical Services (OCS).

6.1.1. Defined in DoWI 6025.13, *Medical Quality Assurance in the Military Health System,* as "clinical and clinical support services on ships and planes, in deployed settings, and in all other circumstances outside an MTF." **(T-0)**

6.1.2. DAF MH OCS are conducted both in garrison and contingency environments.

6.1.3. In garrison, MH OCS is conducted by IOS MH personnel performed outside of the garrison MTF IAW DoWI 6025.13, and DAFMAN 10-211. All MH operations in a contingency environment are considered OCS. **(T-0)**

6.2. Integrated Operational Support (IOS) MH Operations.

6.2.1. IOS MH personnel deliver OCS outside of the MTF in designated OCS locations, to individuals, groups, and leaders aimed to increase operational effectiveness and readiness and to protect and enhance the health of SMs as they execute their mission specific duties. **(T-1)**

6.2.2. At the outset of individual clinical or non-clinical services, IOS providers will inform clients of the IOS scope of support and limits to confidentiality.

6.2.2.1. IOS is an operational readiness capability, not a substitute for standard clinical services. 6.2.2.2. When IOS MH personnel conduct unit surveillance, leadership consultation, or general psychoeducation, these are operational activities and do not constitute clinical encounters. If an interaction transitions into a clinical assessment, or if a SM discloses information meeting the mandatory command notification thresholds of DoDI 6490.08, MHP will document the encounter appropriately and execute required command notifications. **(T-0)**

6.2.3. MDG/CC serves as the installation privileging authority and determines the medical services provided on an installation outside of the MTF. OCS operations will adhere to DoW, DHA, and Service-specific medical policies. **(T-1)**

6.2.4. IOS MH personnel are licensed health care professionals operating under their clinical credentials to enhance operational readiness, whether providing care in the MTF or unit.

6.2.4.1. IOS MH personnel apply psychological health expertise to identify and mitigate occupational stressors, optimize performance, and advise leadership. This role is distinct from and must not be misconstrued as non-clinical "life coaching" or "executive coaching."

6.2.4.2. While IOS personnel may utilize coaching skills, all interventions applied are grounded in evidenced-based methods and their health care professional training, focused on the primary objective of protecting and enhancing the health of the force. **(T-1)**

6.2.5. IAW DoWI 6025.13, IOS MH (OCS) is documented in the ASD(HA) approved EHR or uploaded when practicable and IAW Service guidance. **(T-1)**

6.2.6. When OCS activities conducted by IOS MH personnel exceed low acuity/low risk threshold, as determined by the clinical provider and DAFMAN 10-211, the care will be transitioned to the MTF and managed by IOS provider within the MTF, where possible. **(T-1)**

6.2.7. If member requires a duty or mobility/duty restriction, DES consideration, etc. for a MH disorder, care will be provided in the MTF by the IOS MH provider IAW DHA clinical mental health care policies and procedures. **(T-1)**

6.2.8. When an IOS MH provider is concerned about their ability to maintain safe, quality care for patients, they will report concerns to the MH element chief who will assess and provide support as appropriate. Support includes but is not limited to consultation, case management, or transferring cases to another MH provider. All case management decisions and case disposition will be documented in the EHR. MHPs will not include any medical quality assurance information, privileged by 10 U.S.C. § 1102, in the EHR.

6.2.9. IOS MH personnel will utilize, within the scope of their privileges, appropriate standard of care and evidence-based practices for the conditions being treated, equivalent to the care provided in the MTF. Therapeutic treatment modalities must be evidence-based and align with approved provider privileging and capabilities of the OCS environment.

6.2.10. IOS MH personnel will maintain training requirements and competencies for MH personnel outlined in chapter 3.7.4 of this publication. **(T-1)**

6.2.11. IOS MH personnel are required to follow the same mandatory reporting requirements as providers inside the MTF, IAW DoDI 6490.08, regardless of whether care is rendered. When consulting with unit leadership, IOS MH personnel may only disclose the minimum necessary information to a G-series commander or the commander's designee. **(T-0)**

6.2.12. Providers should avoid SMs having multiple providers between MTF and OCS locations, unless clinically indicated and appropriate. Collaboration among MTF and IOS MH providers will be conducted in the weekly ARTL meeting to ensure IOS providers are aware of members in their unit who are engaged in care at the MTF and to mitigate multiple providers supporting one member. **(T-1)**

6.3. Prohibited MH Services Outside of an MTF.

6.3.1. Some MH services require MTF resources and personnel to provide safe and effective patient care than what is appropriate by IOS MH personnel outside of the MTF. Unless in an austere or contingency environment where no other care location is reasonably available, care will be rendered within the MTF/MH flight. **(T-1)**

6.3.2. The following activities are prohibited outside of the MTF:

6.3.2.1. Treatment of patients at either acute or chronic intermediate or higher risk for suicide or danger to self (e.g., non-suicidal self-injurious behavior). **(T-1)**

6.3.2.2. Treatment of patients at elevated risk for danger to others. **(T-1)**

6.3.2.3. Treatment of current Mental Health ARTL patients. **(T-1)**

6.3.2.4. Treatment of ADAPT patients. **(T-1)**

6.3.2.5. Treatment of FAP patients. **(T-1)**

6.3.2.6. Sanity Boards. **(T-1)**

6.3.2.7. Command-Directed Evaluations, including both emergent and routine. **(T-1)**

6.3.2.8. Treatment of patients with impaired reality testing. **(T-1)**

6.3.2.9. Family Therapy or Marital Therapy. **(T-1)**

6.4. IOS MH Patient and Personnel Safety.

6.4.1. IOS MH personnel will conduct OCS care for non-emergent conditions and issues (low acuity, low complexity, and low risk); nevertheless, OCS MH services will be provided in locations that have safety measures and processes for emergency situations IAW DAFMAN10-211. **(T-1)**

6.4.2. OCS MH will only occur within unit operational hours and only be provided if there are other individuals physically present in the building and available to assist should an emergent situation develop. IOS MH personnel will defer any after-hours acute issues to the after-hours MH consultant and/or the closest ED. IOS MH personnel will address any routine issues the following duty day. When DMH is activated, IOS MH personnel will support the response as appropriate. **(T-1)**

6.4.3. IOS MH personnel operate in a distinct operational environment providing low-acuity services outside of formal MTFs. IOS MH personnel will adhere to a risk-based approach to provider safety according to recommendations from documented safety risk assessment conducted in coordination with SFS.

6.4.4. Safety procedures should include standard communication devices, established check-in/out procedures, and local emergency contact protocols. Due to the low-acuity nature of these services, the installation or utilization of formal, dedicated duress systems is not required. **(T-0)**

6.5. MH OCS in Contingency Environments.

6.5.1. MH personnel serving in contingency environments will, to the extent practicable, adhere to the full spectrum of clinical MH requirements, MHRP, and COSC principles IAW DoDI6490.05. **(T-0)**

6.5.2. MH operations in contingency environments should be similar to garrison MH operations, with variance driven by location, resources, and mission requirements. MH personnel serve as the primary MH resource for command and SMs, providing prevention, command consultation, and intervention based on community and individual needs. **(T-1)**

6.5.3. MHRP is applicable in the contingency environment, and the resources involved will vary based on services available. MH will continue to serve as the DMH lead and all DMH requirements will be conducted IAW DoDI 6200.03 and this instruction. **(T-0)**

6.5.4. COSC principles support the development and maintenance of an environment and climate of prevention and protection. Psychological interventions for combat and operational stress reactions shall be implemented by all SMs and afforded the same importance as addressing physical injury risk in order to mitigate risks of long-term mental and physical health consequences. **(T-0)**

6.5.5. COSC principles enhance adaptive stress reactions, prevent maladaptive stress reactions, build resiliency skills, and assist SMs in addressing stress reactions.

6.5.6. Functional areas of COSC include psychological first aid, needs assessment, consultation and education, triage and stabilization, traumatic event management, treatment/OCS, and restoration.

6.5.7. Main principles of managing combat or acute stress reactions focus on BICEPS:

6.5.7.1. Brevity – shortest intervention necessary

6.5.7.2. Immediacy – as soon as possible

6.5.7.3. Contact – maintain command engagement

6.5.7.4. Expectancy – normalize reactions

6.5.7.5. Proximity – remain close to the unit/front line as possible

6.5.7.6. Simplicity – simplest intervention (rest, food, hygiene, reassurance)

6.5.8. MH operations in contingency environments are designed to optimize the warfighter to remain on duty, support and advise command, and mitigate combat or acute stress reactions. **(T-0)**

Chapter 7

MENTAL HEALTH AND THE LAW

7.1. Limited Privilege Suicide Prevention (LPSP).

7.1.1. LPSP program is designed to facilitate timely MH treatment for Airmen and Guardians who, because of the stress of impending disciplinary action under the Uniformed Code of Military Justice (UCMJ), are considered at risk for suicide. Counseling, therapy or treatment, and admissions can all qualify under this program as outlined in DAFI 51-201, *Administration of Military Justice*. **(T-1)**

7.1.2. Program Eligibility.

7.1.2.1. Any Airman or Guardian who has been officially notified, verbally or in writing, that he or she is under investigation or is suspected of having committed an offense under the UCMJ is eligible for the LPSP program.

7.1.2.2. Any person may inform the SM about the program, including defense counsel and defense paralegals, commanders, First Sergeants, supervisors, and all those involved in the military justice process and encourage the SM to seek treatment.

7.1.3. LPSP protections are initiated upon receiving treatment from an MHP; there is no formal enrollment process by MH personnel for SMs to utilize the LPSP program.

7.1.3.1. MHP are responsible for providing patient care to those eligible IAW DAFI 51-201 and ensuring their patients are aware of LPSP protections.

7.1.3.2. There are no additional administrative or training requirements for MH personnel regarding LPSP.

7.1.3.3. MH should collaborate with local legal office to ensure all stakeholders are properly educating SMs under investigation. MHPs will ensure they are appropriately advocating and educating their patients who are eligible for LPSP. **(T-1)**

7.2. Confidentiality.

7.2.1. General Rule. Communication between a patient and any MH personnel made for the purpose of facilitating diagnosis or treatment of the patient's mental or emotional condition are confidential communications and shall be protected from unauthorized disclosure IAW DoDI 6490.08 and DoDM 6025.18. **(T-0)**

7.2.2. Confidential communications will be disclosed to people or agencies with a proper and legitimate need for the information and who are authorized by law or regulation to receive it, unless the evidentiary privilege applies. **(T-0)** Such disclosures will provide the minimum information needed.

7.2.3. In cases not arising under the UCMJ, the MHP may consult with the DHA Office of General Counsel (OGC) attorney supporting the MTF if they have any questions about releasing the MH records or communications. **(T-1)**

7.2.4. Requests for disclosure of confidential communications by MHP for the purpose of criminal investigation or proceedings under the UCMJ require consultation with the DHA OGC attorney supporting the MTF and the installation SJA advising on UCMJ matters. **(T-1)**

7.3. Military Rule of Evidence 513.

7.3.1. This evidentiary rule provides a SM receiving MH treatment the privilege to refuse to disclose and to prevent any other person from disclosing a confidential communication made between the patient and the psychotherapist or an assistant to the psychotherapist, in a case arising under the UCMJ, if such communication was made for the purpose of facilitating diagnosis or treatment of the patient's mental or emotional condition. **(T-0)**

7.3.2. There is no privilege under this rule in the following conditions:

7.3.2.1. When the patient is dead. **(T-0)**

7.3.2.2. When communication is evidence of child abuse or neglect, or in a proceeding in which one spouse is charged with a crime against a child of either spouse. **(T-0)**

7.3.2.3. When federal law, state law, or service regulation imposes a duty to report information contained in a communication. **(T-0)**

7.3.2.4. When a psychotherapist or assistant to a psychotherapist believes that a patient's mental or emotional condition makes the patient a danger to any person, including the patient. **(T-0)**

7.3.2.5. If the communication clearly contemplated the future commission of a fraud or crime or if the services of the psychotherapist are sought or obtained to enable or aid anyone to commit or plan to commit what the patient knew or reasonably should have known to be a crime or fraud. **(T-0)**

7.3.2.6. When necessary to ensure the safety and security of military personnel, military dependents, military property, classified information, or the accomplishment of a military mission. **(T-0)**

7.3.2.7. When an accused offers statements or other evidence concerning his/her mental condition in defense, extenuation, or mitigation, under circumstances not covered by R.C.M. 706 or Mil. R Evid. 302. In such situations, the military judge may, upon motion, order disclosure of any statement made by the accused to a psychotherapist as may be necessary in the interests of justice. **(T-0)**

7.3.3. Who May Claim the Privilege Under Military Rule of Evidence.

7.3.3.1. The privilege may be claimed by the patient, or the guardian or conservator of the patient. A person who may claim privilege may authorize trial counsel, defense counsel, or any counsel representing the patient to claim the privilege on his or her behalf.

7.3.3.2. The psychotherapist or psychotherapist assistant who received the communication may claim the privilege on behalf of the patient. The authority of such a psychotherapist, assistant, guardian, or conservator to assert the privilege is presumed in the absence of evidence to the contrary. **(T-0)**

7.3.4. Limited Applicability of Military Rule of Evidence 513.

7.3.4.1. The evidentiary privilege only applies when access to confidential information is being sought for the purpose of a criminal investigation or proceeding under the UCMJ.

7.3.4.2. If access is requested for any other purpose (e.g., civilian judicial proceedings), the evidentiary privilege has no application, unless law or service regulation specifically requires its application. **(T-0)**

7.4. Limits of Confidentiality/Informed Consent.

7.4.1. Command notification will be accomplished IAW DoDI 6490.08. **(T-0)**

7.4.2. Command notification by MH personnel is prohibited for protected health information when a SM voluntarily requests MH treatment or substance misuse education, unless the SM authorizes the notification, or the disclosure is authorized for one of the exigent circumstances, as defined in DoDI 6490.08 and DoDM 6025.18. **(T-0)**

7.4.3. Commanders must be notified when a SM meets DoDI 6490.08, Section 3 criteria, including: self-harm, harm to others, mission impairment, special personnel IAW DoDI 5210.42, inpatient care, acute medical conditions affecting duty, substance abuse treatment, CDEs, or situations where mission execution outweighs avoiding notification. **(T-0)**

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Attachment 1

GLOSSARY OF REFERENCES AND SUPPORTING INFORMATION

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Abbreviations and Acronyms

ADAPT—Alcohol and Drug Abuse Prevention and Treatment

AFI—Air Force Instruction

AFMS—Air Force Medical Service

AFMEDCOM—Air Force Medical Command

AFMAN—Air Force Manual

AFPD—Air Force Policy Directive

AFR—Air Force Reserve

AFRC—Air Force Reserve Command

AGAM—Airmen and Guardian Availability Management

ANG—Air National Guard

AMRO—Airmen Medical Readiness Optimization

ARC—Air Reserve Component (Air National Guard and Air Force Reserve)

ART—At-Risk Tracking

ARTL—At-Risk Tracking List

ASIMS—Aeromedical Services Information Management System

AUoF—Arming and Use of Force

CAB—Community Action Board

CFM—Career Field Manager

CAT—Community Action Team

CDE—Commander-Directed Evaluation

CFETP—Career Field Education & Training Plan

COSC—Combat and Operational Stress Control

DAF—Department of the Air Force

DHA—Defense Health Agency

DHN—Defense Health Network

DLC—Duty Limiting Condition

DMH—Disaster Mental Health

DoDI—Department of Defense Instruction

DoW—Department of War

DPH—Director of Psychological Health

DRU—Direct Reporting Unit

ED—Emergency Department

EFMP—Exceptional Family Member Program

EHR—Electronic Health Record

EiP—Employed in Place

EMS—Emergency Medical Services

FAP—Family Advocacy Program

FAPM—Family Advocacy Program Manager

FLDCOM—Field Command

FMRC—Family Member Relocation Clearance

GNSD—Guardian on Non-Sustained Duty

HAF—Headquarters Air Force

HIPAA—Health Insurance Portability and Accountability Act

IAW—In Accordance With

IRILO—Initial Review In Lieu Of

JA—Judge Advocate

LPSP—Limited Privilege Suicide Prevention

MAJCOM—Major Command

MDG—Medical Group

MEB—Medical Evaluation Board

MH—Mental Health

MHC—Mental Health Clinic

MHE—Mental Health Evaluation

MHP—Mental Health Provider

MHT—Mental Health Technician

MOU—Memoranda of Understanding

MRC—Medical Readiness Command

MTI—Military Training Instructor

MTL—Military Training Leader

MTF—Military Treatment Facility

MHRP—Mental Health Readiness Program

MRDSS—Medical Readiness Decision Support System

OCS—Operational Clinical Services

PCM—Primary Care Manager

PH—Psychological Health

PHA—Personal Health Assessment

PHI—Protected Health Information

PRAP—Personnel Reliability Assurance Program

PRP—Personnel Reliability Program

PSP—Presidential Support Program

RILO—Review-in-Lieu-of

SARC—Sexual Assault Response Coordinator

SAPR—Sexual Assault Prevention and Response

SFS—Security Forces Squadron

SG—Surgeon General

SJA—Staff Judge Advocate

SM—Service Member

SPAFORGEN—Space Force Generation

TTM—Treatment Team Meeting

UCMJ—Uniform Code of Military Justice

USAF—United States Air Force

USSF—United States Space Force

Terms

All-Hazards Incident—A threat or an incident, natural or manmade, that warrants action to protect the life, property, public health, and safety of military members, dependents, and civilians at risk, and to minimize any disruptions of installation operations. (Presidential Policy Directive 21 & DoDI 6200.03)

Integrated Operational Support (IOS) —The overarching term of all line funded programs being conducted outside of the Medical Treatment Facility in OCS locations with at least one medical authorization who provides OCS services. (DAFMAN 10-211)

Integrated Operational Support Mental Health—The overarching term of all line funded MH personnel and programs conducted outside of the Military Treatment Facility in OCS locations providing OCS services. (DAFMAN 10-211)

Mental Health Provider (MHP)—Military (Active or Reserve component) and civilian personnel (Civil Service and other providers working under contractual or similar arrangement) granted privileges to diagnose medical conditions and initiate, alter or terminate health care treatment regimens within the scope of his or her license, certification or registration.

Mental Health Personnel—Any professional working within the mental health career fields with generally similar privileges, practice, specialty, and level of training. Professions include but are not limited to psychiatrists, psychologists, clinical social workers, psychiatric nurse practitioners, mental health nurses, mental health case manager, and mental health technicians.

Mental Health Professional—An appropriately credentialed clinical psychologist, psychiatrist, licensed clinical social worker, or psychiatric advanced practice registered nurse. This is an all-in inclusive list of professions. (DAFI 36-3211)

Mental Health Services—The comprehensive portfolio of all prevention, assessment, treatment, readiness and consultation activities provided DAF MH personnel.

Mental Health Treatment—The specific set of planned, evidence-based clinical interventions, such as psychotherapy or medication management, provided to a patient following a formal diagnosis to address a mental health condition. All treatment is documented in the EHR.

Operational Clinical Services (OCS)—Clinical and clinical support services on ships and planes, in deployed settings, and in all other circumstances outside an MTF. (DoDI 6025.13)

Postvention Response Team (PRT) —A group of professionals who are trained and prepared to act immediately after a suicide death. (DTM 25-004)